



Situation Analysis

SUDAN CRISIS

Period: 11/05/26 – 17/05/26

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Introduction

Purpose

This report provides a comprehensive overview of the current humanitarian situation, focusing on both operational environment factors and sectoral analysis. It aims to support informed decision-making for humanitarian partners and stakeholders.

About GANNET

GANNET is a Generative AI Initiative from Data Friendly Space, offering AI-powered tools built to make knowledge and real-time information accessible to our community in a resource-efficient and ethical way. It provides tools such as the GANNET Virtual Assistant and GANNET SituationHub to help humanitarian actors access, analyze, and utilize critical information for their operations.

GANNET SituationHub

The GANNET SituationHub is a generative AI-powered digital public good designed to identify and analyze humanitarian data from thousands of trusted sources in real time. By transforming complex data into clear insights, it enables faster, more effective response strategies and improved coordination among stakeholders.

GANNET Virtual Assistant

The GANNET Virtual Assistant is a verified chatbot designed for humanitarian quick decision-making. Built on cutting-edge Generative AI and trained with humanitarian expert-reviewed data, it provides real-time information. The GANNET Virtual Assistant facilitates rapid data access and synthesis for crisis response.

Methodology Overview

The Situation Hub's analytical process uses a structured Human-in-the-Loop workflow that combines automated analysis retrieval through GANNET Virtual Assistant, DFS's AI-powered humanitarian assistant, with expert review by the DFS analysis team. GANNET automates the extraction and generation of crisis-related insights based on structured prompts, which are aligned with a tailored analytical framework. These outputs are then reviewed, validated, and refined by analysts to ensure accuracy, contextual relevance, and operational utility. The workflow is organized into four key phases: planning and setup, ongoing operations, quality assurance, and system evolution, ensuring a balance between speed and analytical rigor.

Planning, Setup & Quality Control

In the planning phase, analysts develop a contextualized analytical framework, largely based on the Joint Intersectoral Analysis Framework (JIAF), and convert it into structured prompts for AI-assisted data extraction. These prompts are refined over time to improve output quality. Data sources are vetted for reliability, and translations are reviewed for accuracy. Using a centralized Content Management System (CMS), analysts review and curate AI-generated outputs, validate them against external sources, and make refinements to enhance clarity and relevance. The CMS enables collaborative editing and version tracking, ensuring outputs align with humanitarian needs.

Bias Mitigation

To minimize bias, the team diversifies data sources, cross-verifies information, and continuously evaluates the AI's performance. Analysts are crucial in identifying and correcting inaccuracies, especially in automated categorization. Future improvements aim to enhance source credibility checks and sentiment analysis, further strengthening the reliability of the Situation Hub's outputs.

Crisis Overview

The SituationHub landing pages use a standardized yet customizable template to deliver consistent, comprehensive crisis overviews. These pages integrate static content with dynamic data from sources like ACLED and DTM via open APIs, enabling interactive visuals with real-time updates. Regular reviews and verification with in-country partners ensure accuracy, while manual updates are used for sources not connected to APIs.

Media Monitoring

The media monitoring process has evolved from a manual, list-based approach to an AI-assisted system integrated into the CMS. This allows analysts to efficiently review, filter, and contextualize media narratives, while automation helps track emerging trends.

Overview

The following overview has been generated using the information available up to May 25, 2026. It provides a synthesized summary and key insights into the crisis based on the most recent data accessible at that time.

Summary

Sudan is facing one of the world's largest humanitarian and displacement crises as the conflict between the Sudanese Armed Forces (SAF) and the Rapid Support Forces (RSF), which began in April 2023, enters its fourth year in 2026. The conflict has triggered widespread civilian suffering, economic collapse, mass displacement, and severe regional instability, with humanitarian conditions continuing to deteriorate across much of the country.

An estimated 19.5 million people are currently experiencing acute food insecurity, making Sudan the world's largest hunger crisis. Famine conditions have been confirmed in el-Fasher (North Darfur) and Kadugli (South Kordofan), while numerous areas across Darfur and Kordofan remain at high risk of famine amid ongoing conflict, siege tactics, disrupted markets, and severe restrictions on humanitarian access. Children continue to bear a disproportionate burden of the crisis, with millions exposed to acute malnutrition, disease outbreaks, and heightened mortality risks.

Fatality estimates remain highly contested due to limited humanitarian access, communication blackouts, and verification constraints. The Armed Conflict Location & Event Data Project (ACLED) recorded nearly 30,000 reported deaths by late 2024, while several independent investigations and international media estimates suggest the true death toll may exceed 150,000 people. Civilians continue to face widespread violence, including indiscriminate shelling, aerial bombardments, drone attacks, and ethnically targeted killings, particularly in Darfur and parts of Kordofan.

Sudan's health system remains severely degraded, especially in active conflict zones where many facilities are non-operational or functioning only partially. Between 2024 and early 2026, Sudan experienced a nationwide cholera outbreak that spread across all 18 states, infecting more than 124,000 people and causing over 3,500 deaths before authorities declared the outbreak contained in March 2026. However, overcrowded displacement sites, poor sanitation conditions, and limited healthcare access continue to create significant risks of renewed outbreaks of cholera and other communicable diseases.

Displacement continues to rise at an unprecedented scale. More than 14 million people have been displaced since the start of the conflict, including approximately 9 million internally displaced persons (IDPs) and more than 4 million refugees who have fled to neighboring countries, making Sudan the world's largest displacement crisis. At the same time, limited returns to parts of Khartoum and Al Jazirah have increased since early 2026 as frontlines shifted, although returnees continue to face devastated infrastructure, insecurity, limited public services, and severe livelihood shortages.

Militarily, the conflict has continued to expand and fragment since late 2025. The RSF has expanded its territorial influence across much of Darfur and intensified offensives in Kordofan, while the SAF has maintained control over key eastern and northern urban centers. Fighting around Kadugli, Dilling, and other strategic locations in Kordofan has trapped large civilian populations under increasingly dire humanitarian conditions. The conflict has also seen a growing use of drones, aerial strikes, and long-range attacks targeting civilian infrastructure, including hospitals, schools, markets, and displacement sites. In December 2025, a drone strike on a kindergarten and hospital in Kalogi reportedly killed at least 114 people, including dozens of children, while separate attacks on peacekeeping personnel highlighted the increasing risks faced by humanitarian actors and civilians alike.

Ethnic violence and sexual and gender-based violence (SGBV) continue to escalate, particularly in Darfur. Human rights organizations and women-led monitoring networks have documented widespread abuses, including conflict-related sexual violence, forced displacement, arbitrary detention, and attacks targeting ethnic communities. Despite sustained diplomatic pressure, including sanctions imposed by the United States and United Kingdom on RSF leaders and affiliated financial networks, regional and international mediation initiatives have thus far failed to secure a durable ceasefire or political settlement.

The conflict is increasingly destabilizing neighboring countries through refugee flows, cross-border insecurity, arms trafficking, and growing pressure on already fragile humanitarian systems across the region. Meanwhile, humanitarian operations remain critically underfunded. The World Food Programme (WFP) warned in 2026 that severe funding shortfalls threaten additional ration cuts and disruptions to emergency food assistance pipelines, placing millions at further risk of hunger and malnutrition.

As of mid-2026, Sudan remains trapped in a protracted and increasingly fragmented conflict characterized by territorial fragmentation, widespread civilian targeting, deepening food insecurity, and severe humanitarian access constraints. Without urgent, coordinated, and sustained international engagement—including increased humanitarian financing, civilian protection measures, and renewed diplomatic efforts toward a negotiated settlement—the crisis is likely to continue worsening, with profound implications for Sudan and the wider region.

Key Insights

1. Sudan's conflict between the Sudanese Armed Forces (SAF) and the Rapid Support Forces (RSF) has created the world's largest displacement crisis. Since the conflict began in April 2023, more than 14 million people have been forcibly displaced, including approximately 9 million internally displaced persons (IDPs) across all 18 states and more than 4 million refugees who have fled to neighboring countries including Chad, South Sudan, Egypt, Ethiopia, Libya, and the Central African Republic. The International Organization for Migration (IOM) continues to describe Sudan as the world's largest internal displacement crisis, with displacement patterns remaining highly fluid as fighting intensifies across Kordofan and violence persists in Darfur. Continued insecurity, deteriorating humanitarian conditions, and repeated attacks on civilian areas are driving ongoing population movements both within

Sudan and across regional borders.

2. A catastrophic humanitarian situation persists in Sudan, with nearly 25 million people—more than half the population—experiencing acute food insecurity. Famine conditions have been confirmed in el-Fasher (North Darfur) and Kadugli (South Kordofan), with at least 20 additional districts across Darfur and Kordofan at serious risk of sliding into famine. Global acute malnutrition exceeds 15% in several areas and surpasses 30% in parts of Darfur and Kordofan—above IPC Phase 5 famine thresholds. An estimated 825,000 children are projected to suffer severe wasting in 2026. Simultaneously, the health system has effectively collapsed: 70% of facilities are non-functional, and cholera has spread to all 18 states with over 113,000 cases and 3,000 deaths since 2024.

3. The conflict has expanded significantly in December 2025. The RSF has consolidated control over most of Darfur following el-Fasher's fall in late October and is now advancing east into Kordofan. In early December, the RSF reportedly seized the SAF's 22nd Division base in Babanusa and captured the strategic Heglig oil field on December 8, halting oil production and threatening state revenues. Cities including Kadugli and Dilling are now besieged. The SAF retains control of the north, east, and center—including Khartoum and parts of Kordofan—but faces sustained pressure as the RSF mobilizes to seize further territory and strategic resources.

4. Sudan is facing multiple disease outbreaks across all 18 states, with cholera, malaria, and dengue widespread. Over 113,000 cholera cases and 3,000 deaths have been recorded since 2024, with outbreaks concentrated in overcrowded displacement sites where water and sanitation systems have collapsed. Malaria has reached epidemic proportions, with Sudan accounting for 41% of global cases. The collapse of water supply systems and sanitation services continues to worsen the public health crisis.

5. Humanitarian operations face severe challenges as access constraints and funding shortfalls intensify. WFP has warned of imminent pipeline breaks and plans to cut rations from January 2026—reducing to 70% in famine areas and 50% elsewhere—with full pipeline breaks possible by April 2026 without an urgent \$695 million injection. The 2025 HNRP requiring \$4.2 billion remains only about 25% funded. Despite these constraints, humanitarian actors reached 1.6 million people in Khartoum between January–June 2025 and continue life-saving operations, while approximately 2.6 million returnees have been recorded across stabilized areas.

6. Civilian harm has escalated dramatically in December 2025. A devastating drone strike on a kindergarten and hospital in Kalogi, South Kordofan, on December 4 killed at least 114 people, including 63 children. A separate drone attack killed six UN peacekeepers. The UN Human Rights Office documented at least 269 civilian deaths in Kordofan since October 25. WHO has recorded 63 attacks on healthcare in Sudan in 2025, resulting in 1,611 deaths and 259 injuries. Sieges and starvation tactics are being used as weapons of war, with an estimated 70,000–100,000 people trapped in el-Fasher under famine conditions.

7. Systematic sexual and gender-based violence continues at alarming levels. The Strategic Initiative for Women in the Horn of Africa (SIHA) verified 1,294 SGBV incidents across 14 states since April 2023, finding that rape accounted for more than three-quarters of incidents, 225 cases involved children as young as four, and where perpetrators were identified, 87% were attributed to RSF. Patterns include initial home invasions with rape, public-space attacks, and long-term detention with gang rape, torture, and forced marriage—with victims from non-Arab communities including Masalit, Fur, and Zaghawa directly targeted. These violations have been characterized as systematic, warranting investigation for war crimes and crimes against humanity.

8. International pressure on conflict parties has increased. In December 2025, the US imposed sanctions on a transnational network recruiting Colombian mercenaries for the RSF, while the UK sanctioned senior RSF commanders and pledged £21 million in emergency assistance. A US-led Quartet (US, Saudi Arabia, UAE, Egypt) has proposed a three-month humanitarian truce followed by a permanent ceasefire and civilian transition, though the SAF rejected the proposal while RSF declared a unilateral ceasefire that has not held on the ground. UN-led Geneva talks are planned, but prospects for a negotiated settlement remain dim amid continued fighting and external support to warring parties.

9. Sudan is experiencing rapid fragmentation of state authority, with parallel and competing governance structures emerging across SAF- and RSF-controlled territories. While the SAF retains formal control over parts of Khartoum, the north, and eastern Sudan, and the RSF exercises de facto authority across most of Darfur and expanding areas of Kordofan, national-level governance has largely broken down. Public administration, taxation systems, justice mechanisms, and service delivery have been severely disrupted or replaced by localized and often militarized governance structures. This fragmentation has weakened national coordination of humanitarian response, complicated negotiations over access, and deepened regional disparities in protection and service provision. As a result, civilians are increasingly governed through fragmented, militarized authorities rather than a unified state system, accelerating institutional collapse alongside the ongoing conflict.

Economy

Current economic situation

Conflict has sharply contracted economic activity, particularly around Khartoum, with the agrifood system's GDP contribution estimated to have fallen 21% in 2023, about 73% of food-processing firms permanently or temporarily closed in 2023, and agriculture still accounting for roughly 35% of GDP and 40% of employment in 2023 [1]. National cereal production for 2025/26 was estimated at 5.2 million tonnes – 22% below last year and 27% below 2022/23, with a 2.8 million tonne deficit – while staple and cash-crop outputs and incomes declined amid input shortages and access constraints, and sorghum, millet and wheat prices remain 270–430% higher than March 2023 in many markets [12]. Bank closures, market disruptions and soaring transport costs are constraining domestic liquidity, trade and food access across affected areas, while goods-transport bans into RSF-held areas and the Chad border closure since mid-March (with limited humanitarian exceptions) are tightening supply lines and restricting commodity volumes reaching local markets [23][12][4]. The Port Sudan–Kassala–Gedaref corridor – previously handling roughly 90% of Sudan's international trade – remains a critical vulnerability as drone strikes and attacks on fuel or port infrastructure threaten to disrupt imports and humanitarian supplies [23]. Persistent funding shortfalls are amplifying macro and humanitarian stress: only about 20% of Sudan's 2026 Humanitarian Needs and Response Plan had been funded as of April 2026, limiting relief that could stabilize markets and protect livelihoods [28]. UNDP reported that poverty has doubled since the conflict began, with about 70% of the population below the poverty line and one in four in extreme poverty [10].

Impact of the conflict on the economy

Acute fuel and logistics shocks have compounded the economic crisis: parallel-market gasoline in Khartoum surged nearly 67% in a single week in early April 2026, black-market gasoline remained about 36% above year-on-year levels, and shipping disruptions linked to the Middle East crisis added up to 25 days to deliveries to Sudan, further delaying imports [7][12][9]. These fuel shocks have raised transport costs, forced substitution of trucks with motorcycles and animal carts, and slowed commercial flows. Around 54% of fertilizer imports originate from Gulf countries, making planting and seasonal market supply highly vulnerable to regional shipping and sanctions shocks, and raising costs that threaten the upcoming planting season [7]. Currency depreciation, high inflation and deteriorating terms of trade between February–May 2026 are eroding purchasing power and market access [4][28][15]. Bureaucratic impediments, a dual governance environment and politicisation of humanitarian registration and operations are further limiting commercial and aid flows, constraining market recovery and humanitarian response in Darfur and Kordofan [4][21][23].

Stability of the currency and banking systems

Currency depreciation and inflation have sharply reduced the purchasing power of humanitarian cash assistance: by March 2026 transfers covered only 59% of a standard food basket and, after the April 2026 fuel shock, Mercy Corps estimated this fell to approximately 43% – representing a roughly 57% collapse in purchasing power [7][4][15]. WFP monitors both official and parallel exchange rates in Sudan and reports widening official–parallel gaps; Sudan is noted among countries where parallel exchange rates are actively estimated by country offices, underlining reliance on parallel-market dynamics for currency valuation in operational planning [9]. IPC and joint WFP/FAO/UNICEF analyses link currency depreciation and inflation to rising food and fuel prices, disrupted supply chains, and constrained humanitarian and commercial market access, exacerbating household food insecurity [4][28][15].

Employment

The Sudan Strategy Support Program (IFPRI, May 2026) found that engagement in agricultural activities has dropped: household incomes have contracted, cultivated land and livestock holdings have declined, and households increasingly rely on non-agricultural businesses, casual labour and humanitarian assistance for livelihoods [1][18]. The report recommends restoring market access, inputs and rural infrastructure to revive farming-related employment and incomes [1]. FEWS NET and recent humanitarian updates (March–May 2026) highlight severely constrained income-earning opportunities, disrupted markets, rising fuel and transport costs, and a collapse in the purchasing power of cash transfers, all of which limit job availability and the effectiveness of cash-based assistance [12][7]. IOM noted that providing jobs and safe incomes for vulnerable families and returnees is a priority amid large-scale returns and chronic underfunding of shelter and non-food assistance [22].

Politics

Overall political landscape

Sudan's political landscape is defined by the coexistence of two competing governance structures. The RSF-led TASIS accelerated the creation of parallel state structures in May 2026, announcing new leadership and specialized committees on 14 May and completing appointments to executive and oversight roles in areas under its control [143]. A separate Tasees administration headquartered in Nyala has moved to establish administrative institutions, forming an independent police force alongside civil registry and judicial functions [119][114]. Observers flag that the formation of security and policing bodies by actors accused of arbitrary detention, extortion, and documented grave abuses raises fundamental questions about legal oversight, accountability, and the effectiveness of the rule of law in areas under parallel administrations [119][146].

The institutionalization of these parallel structures has directly complicated humanitarian access. TASIS's National Humanitarian Access Authority extended an NGO registration deadline to 13 June and reported that 36 of 43 international organisations and approximately 138 national organisations operating in TASIS areas had complied with registration procedures [174].

The army-controlled Ministry of Foreign Affairs warned that organisations registering with TASIS would be treated as supporting parallel entities, a stance humanitarian actors and the World Food Programme say politicises aid and risks constraining access across contested areas [114]. The UN Office for the Coordination of Humanitarian Affairs and UNICEF warned that insecurity, including drone strikes, is obstructing humanitarian access, and reported that over 15,000 people in Um Baru locality are expected to receive assistance amid malnutrition rates that have exceeded emergency thresholds [155][164].

High-profile defections and internal splits within RSF-aligned networks — exemplified by West Darfur native administration leader Masar Asil's public shift to the Sudanese army on 12 May — reflect ongoing fragmentation and contested legitimacy within informal power structures [179].

Political stability

Political stability in Sudan is severely undermined by the institutionalization of competing authorities and their direct impact on humanitarian operations. The emergence of TASIS and its humanitarian bodies has created parallel governance structures requiring NGOs to register and open offices in RSF-controlled areas, effectively politicising aid delivery [114][147][174]. On 15 April, the National Humanitarian Access Authority gave organisations one month to register and ordered UN agencies to reopen offices in RSF areas within 45 days, prompting fears of withdrawals and contingency planning among foreign NGOs by early May [147]. Authorities threatened removal of non-compliant organisations, formed committees to inventory assets of organisations refusing to register, and some organisations already suspended activities in parts of Darfur amid mounting uncertainty [147]. Reports that 160 WFP-linked trucks carrying 6,194 tonnes of aid were being held further illustrate how competing controls are obstructing aid flows on the ground [109].

The combined effect of armed-actor fragmentation, competing territorial controls, parallel checkpoints, and administrative hurdles has substantially increased bureaucratic impediments and restricted humanitarian movement [134][114]. These dynamics have deepened an already acute funding and delivery gap: only approximately 20% of Sudan's 2026 Humanitarian Needs and Response Plan had been funded as of April 2026, and food assistance reached an estimated 3.13 million people in February against a monthly target of roughly 4.8 million [100][101][115]. The US designation of the Sudanese Muslim Brotherhood and sanctions targeting BBMB on 16 March 2026 added further external pressure, complicating the political alignments of SAF-allied networks [134].

Elections

Civil society commentary on 13 May 2026 advocated a tripartite election on new terms as the quickest route to restore popular legitimacy and regional-central inclusion, warning that failure to hold genuinely consultative polls fuels the crisis [158]. The British Parliamentary Committee's refusal to host Sudan's de facto Prime Minister Kamel Idriss signalled international concern about engaging figures tied to the 25 October 2021 coup, and about traditional parties — including Jaafar Al-Mirghani's original federal party and Abdulrahman Sadiq al-Mahdi's National Umma Party — seen by critics as having obstructed the civilian transition [172].

Several developments illustrate the structural obstacles to credible electoral processes. The RSF-led TASIS parallel authorities' moves to institutionalize de facto administrative control — including compulsory NGO registration — entrench competing governance and complicate a neutral electoral environment [174][147]. The Khartoum government's use of nationally significant instruments, such as politicizing national school examinations to showcase state functionality, has been employed as a political tool that undermines perceptions of an impartial state necessary for credible elections [135]. Ongoing fragmentation among armed actors, exemplified by RSF field commander Ali Rizq Allah publicly declaring his defection on 11 May 2026, highlights fluid non-state military influence over territory and constituencies that would impede inclusive, secure electoral processes [201][166].

Political stakeholders

The primary political stakeholders are the Sudanese Armed Forces (SAF), the Rapid Support Forces (RSF), and a range of allied armed movements including JEM, Minni Arko Minawi's faction, BBMB, and SLM-Northern [134][139][114][119]. SAF depends operationally on allied armed groups, but efforts to integrate or constrain them have stalled, increasing the risk of fragmentation and localised confrontation [134]. The Sudanese army also functions as a major economic actor and patronage hub with interests in gold, enterprises, and trade networks, while tribal loyalties and clan networks are actively mobilised by both sides to control territory and resources [110][122][166].

RSF-aligned authorities have built parallel institutions — including a chief justice, Council of Regions, ministers, and a separate police force — to contest Khartoum's legitimacy [114][119]. The RSF's strong ties to Arab tribes in Darfur and repeated allegations of ethnically targeted violence have intensified local fear and prompted international calls for accountability [166][126]. High-profile defections from RSF to the army — driven by military intelligence operations and monetary incentives — were reported in May 2026, including the confirmed defection of Brig. Ali Rizqallah on 11 May and West Darfur native administration leader Masar Asil declaring support for the army on 12 May [166][179].

External regional actors and transnational arms and finance flows are reshaping patron-client dynamics. A 5 May 2026 airport strike — attributed by the army to Ethiopia — and ongoing Gulf and regional diplomacy illustrate how external pressure can temporarily shift internal alliances [133][139][171]. The combined effect is a weakening civilian political sphere and the emergence of multiple de facto centres of authority where military, paramilitary, and allied movements exercise governance through patronage networks rather than functioning national institutions [132][118][114].

Historical barriers to power

Colonial-era extraction, artificial borders, and a Khartoum-centred state since independence have concentrated authority and resources in the centre, producing long-term marginalisation of peripheral regions and communities [159][137]. The military evolved into a dominant political and economic actor — controlling mining, companies, and rent networks since the 1980s — subordinating civilian institutions and narrowing civilian access to power [110][125].

Closed patronage networks and restricted elite circulation have excluded Darfur, Kordofan, Blue Nile, and other marginalised areas from meaningful political participation, deepening grievances and fragmentation [159][125][134].

The 2019 political breakdown and the subsequent SAF–RSF war escalating from 2023 accelerated state fragmentation and the emergence of parallel authorities, further blocking unified, inclusive political channels [127][132][119]. Humanitarian and risk analyses in 2026 highlight that militarisation, large-scale displacement, politicisation of services, and RSF mobilisations near Al Obeid since October 2025 are increasing the targeting and exclusion of marginalised communities and complicating their access to decision-making and aid, thereby reinforcing historical barriers to participation [134].

Trust in institutions

Public trust in Sudan's state institutions is severely eroded as competing de facto authorities build parallel institutions that directly challenge central legitimacy [119][114]. Security forces — primarily SAF and the RSF — have become the main sources of political, administrative, and economic support, with the army controlling major economic sectors and security organs exercising de facto governance roles [110][114][118]. Systemic abuses — including arbitrary detention, enforced disappearances, and targeted repression of journalists, lawyers, and other professionals — have delegitimised the justice system and deepened popular mistrust [181].

The politicisation of state functions is visible in the competing registration demands placed on humanitarian organisations and the mutual warnings issued by both parallel authorities, undermining neutrality and public confidence in institutions [114]. The Port Sudan authority has used nationally significant instruments, such as organising Sudanese certificate examinations, to signal continuity and bolster its claim to state legitimacy [135]. The Attorney-General's establishment of a National Commission to investigate wartime crimes is presented as a step toward accountability and restoring trust in legal institutions [146]. Humanitarian safeguarding actors highlight a pronounced trust deficit — marked by low reporting of sexual exploitation and abuse, weak accountability, and reputational risks — warning that impunity and ineffective safeguards will further erode community trust in both state and aid actors [157]. Analyses frame this as a structural institutional crisis in which military and security institutions dominate state functions, diminishing civilian governance and public confidence overall [132][110].

Security & Stability

Security overview

Conflict intensity has escalated with a sharp increase in drone strikes driving a "deadlier" phase of the war, with drone strikes becoming the leading cause of civilian deaths between January–April 2026, according to the UN human rights office and Protection Cluster assessments [282][231]. At least 880 civilian deaths from drone strikes were recorded in that period, accounting for the majority of reported conflict fatalities [282]. The Protection Cluster reports nationwide, systematic attacks on civilians and civilian objects — including markets, hospitals, schools, IDP sites, and water infrastructure — alongside widespread siegelike conditions, movement restrictions, pervasive looting, forced evictions and discrimination that together amplify civilian harm and impede humanitarian access [231][224]. Protection threats highlighted across sources include indiscriminate aerial and drone strikes, siege and forced displacement, genderbased violence, forced recruitment of children, extortion and occupation of civilian property, and targeting or obstruction of humanitarian actors and assets [231][224][245].

ACAPS' 13 May 2026 risk analysis flags high or medium risk of intensified hostilities and humanitarian deterioration on multiple fronts — notably North Kordofan (Al Obeid), West Kordofan, Blue Nile, and disruptions to Red Sea and Port Sudan supply corridors — raising the likelihood of expanded displacement and access constraints between June–December 2026 [233][245]. Fragmentation within and between armed actors is increasing: efforts by RSFlinked authorities to build parallel institutions (including a separate police force) and signs of SAF alliance tensions raise risks of local power contestation, entrenchment of armed control, and further erosion of civilian protection and rule of law [252][245]. The conflict is also regionalising, with repeated crossborder strikes and displacement; Chad reported repeated drone/missile strikes on Sudanese border markets and official border closures since February, with 926,421 registered Sudanese arrivals in Chad as of April 2026, straining host communities and raising crossborder protection risks [246][229].

Humanitarian access remains severely constrained by active hostilities, sieges, road blockages and bureaucratic impediments that restrict deliveries, reduce data collection and undercount civilian harm, while attacks on health and other essential infrastructure have rendered many services nonfunctional [236][231]. Key figures illustrating the scale of humanitarian impact include:

- Nearly 5.6 million people projected to face IPC Phase 3+ acute food insecurity between June–September 2026 in areas with available data [236].
- An estimated ~40% of health facilities nonoperational as of January 2026 [236].
- An estimated 825,000 children under five projected to suffer severe acute malnutrition in 2026, exacerbating protection and mortality risks [236].

International calls for deescalation and protection of civilians intensified in May 2026, with the UN SecretaryGeneral and the UN Personal Envoy urging an immediate cessation of hostilities, tighter controls on arms transfers and unhindered humanitarian access [287][288].

Security forces and law enforcement

National security forces are functionally fragmented among the Sudanese Armed Forces (SAF), the Rapid Support Forces (RSF), allied armed groups and local militias, producing parallel control structures and multiple checkpoints that have weakened centralized law enforcement and risk further fragmentation [245][233].

RSFlinked moves to form a separate police body ('Tasees') have raised specific concerns about militarizing policing, creating parallel sovereign institutions, and the absence of independent oversight and accountability, amid documented allegations of arbitrary detention, extortion and mistreatment by RSF-affiliated forces [252]. Armed-party territorial control and fragmentation have sharply constrained law-enforcement and humanitarian presence in Darfur, Kordofan and parts of Khartoum State, limiting response capacity and increasing impunity [241][245][253].

Recruitment and forced conscription (including of children), arbitrary detention, looting, extortion and restrictions on movement by armed actors are widely reported and are undermining civilian protection and rule of law [245][224]. Explosive ordnance contamination — including confirmed landmines, UXO and ERW — has risen in urban and rural areas, impeding freedom of movement, access to services and law-enforcement operations [224]. Humanitarian and law-enforcement operational capacity has been further degraded by attacks on aid workers, abductions and targeted strikes on convoys, warehouses and hospitals, with fatalities and kidnappings among aid personnel reported in 2025–2026, reducing oversight and accountability on the ground [241][245].

Risks affecting social stability

Intensified intercommunal and tribal clashes in Darfur and Kordofan — including deadly confrontations that burned homes and prompted temporary local ceasefires — and violent competition over scarce water and resources across borders (notably clashes in Chad causing dozens of deaths) are increasing localized criminal and communal violence [232][246]. Fragmentation among armed groups and within the SAF–RSF axis is driving forced recruitment, looting, ethnic targeting and territorial fragmentation, complicating security dynamics and heightening risks of localized criminal violence and vigilantism [245].

These dynamics are driving mass displacement — close to nine million people internally displaced within Sudan as of endMarch 2026 [220][222][246] — straining host communities and raising the potential for tensions and unrest in reception areas. Attacks, siege-like tactics, restricted movement and bureaucratic impediments are severely limiting humanitarian access and operations, while the 2026 Humanitarian Response Plan remained only approximately 20% funded in April–May 2026, reducing capacity to prevent or mitigate unrest stemming from unmet needs [220][222][219]. Humanitarian actors are increasingly dependent on local responders for access negotiation and alternative logistics — including ICRC's RED aircraft hub in Jeddah to sustain staff deployments — but these adaptations do not remove the heightened risk of criminal violence, protection abuses or community unrest [245][299].

Legal & Policy

Legal and policy framework

Sudan is party to core international legal instruments relevant to humanitarian response and human rights, having ratified the four Geneva Conventions of 1949 (and the two Additional Protocols in 2005), the African Charter on Human and Peoples' Rights (18 February 1986), and the International Covenant on Civil and Political Rights (18 March 1986) [308]. Legal assessments also note the absence of a permanent Sudanese constitution, referencing instead a transitional constitutional document amended on 15 February 2025, a factor affecting national guarantees for human rights and rule-of-law measures in the current context [308]. Sudan is additionally a State Party to the Chemical Weapons Convention, and participants at the African Commission forum in Banjul (7–9 May 2026) urged Khartoum to fully cooperate with international investigations, allow OPCW technical teams to deploy amid allegations of chemical-weapons use, and publish the findings of a national investigative committee established in July 2025 [344][339].

The UN Human Rights Council created an Independent International Fact-Finding Mission on Sudan in October 2023 (mandate extended October 2025), and joint AU–UN fact-finding teams issued the Banjul Joint Declaration during the African Commission's 87th session (24 April–20 May 2026), calling for an immediate ceasefire, independent investigations, and an international accountability pathway after finding grave violations by parties to the conflict, with abuses by the RSF described as particularly widespread and systematic [346][321][338]. The joint investigators called for international and African accountability paths, including cooperation with the ICC and establishment of African-led mechanisms [338][315]. The Sudanese delegation formally rejected the Banjul Declaration, arguing the document lacked legal basis and calling for investigations to include national mechanisms and in-country victim interviews [357][348][346][338].

AU–UN communiqués and humanitarian analyses in May 2026 emphasised the need for an inclusive inter-Sudanese dialogue, adherence to International Humanitarian Law, protection of civilians, and removal of administrative and security barriers to enable safe, sustained humanitarian access [369][312][354]. UN/coordination PSEA analyses (January 2026) report increasing government engagement on Protection from Sexual Exploitation and Abuse and recommend institutionalising safeguarding through codes of conduct, HR policies, and survivor-centred referral pathways, with River Nile government engagement cited as a replicable model [303][332].

Humanitarian impact of legislation

Competing authorities have deeply politicised humanitarian registration and access. The RSF-led "Tasis" directive requires international organisations to register with it and open headquarters in Nyala to operate in RSF-controlled areas, while the army-controlled Ministry of Foreign Affairs warned that any engagement with parallel entities would violate sovereignty — creating threats of expulsion and delaying visas that hinder operations, particularly in Darfur and Kordofan [311]. ACAPS warned in mid-May 2026 that fragmentation among SAF and allied armed groups could force responders to negotiate access with multiple local armed actors and tighten territorial control, heightening humanitarian movement constraints and access barriers [310]. External political and military support to conflict parties further complicates neutral aid delivery and weakens enforcement of humanitarian norms, politicising access and recognition decisions that impede countrywide operations [311][381][359].

Protection Cluster reporting documents widespread bureaucratic impediments, siege-like conditions, checkpoints, and restrictions on freedom of movement that deny civil documentation and obstruct aid, exacerbating exclusion of IDPs, returnees, and other vulnerable

groups [320][323]. UN agencies and the WFP/FAO/UNICEF joint statement noted severe humanitarian access constraints — including attacks on supply routes, destruction of civilian infrastructure, and administrative barriers — which have reduced the scale, timeliness, and coverage of assistance [304][324]. Government enforcement of residency laws has included deportations of South Sudanese, with official authorities asserting compliance with national legal provisions even as humanitarian monitors report higher numbers and family separations [363][364].

As a result of these compounding barriers, operational reliance has shifted toward community-embedded civil society, mutual aid groups, and local actors better able to negotiate access and operate under administrative restrictions [310].

International agreements impacting the crisis

The AU and UN reaffirmed strategic partnership and called for inclusive inter-Sudanese dialogue and coordinated international action, referencing implementation of UN and AU frameworks for peace and support to AU-led efforts such as those linked to UNSC resolutions on peace operations [369][354]. UN Secretary-General António Guterres urged urgent international action and an immediate cessation of hostilities on 12 May 2026 to prevent regional destabilization and alleviate humanitarian suffering [349]. The U.S. House Foreign Affairs Committee approved a draft resolution condemning attacks on civilians, attributing documented war crimes to both SAF and RSF, and calling for measures to end external material support to the parties and to advance accountability [335].

International pledging conferences, including the Berlin conference, mobilized substantial commitments — reported near €2 billion in pledges — but humanitarian funding remained critically short, with only approximately 20% of Sudan's 2026 Humanitarian Response Plan funded as of April 2026, constraining operational responses [301][322][304][305].

Limitations of current policies

Existing legal and policy frameworks face significant structural and operational shortcomings. The January 2026 PSEA SWOT analysis found that PSEA remains weakly institutionalised and largely driven by external mandates rather than embedded government systems, limiting sustainability, localisation, and survivor-centred outcomes [303][332]. The analysis further warns that narrowing international access, accelerated localisation, economic collapse, and the arrival of new humanitarian actors without adequate oversight amplify risks of sexual exploitation and abuse, while reporting and oversight channels are constrained by insecurity [303].

Humanitarian actors report pervasive bureaucratic impediments — visa delays, dual governance disputes, and administrative restrictions — that constrain access, delay programming, and reduce coverage, especially in Greater Darfur and Kordofan [311][304][312]. ACAPS notes that existing policies and coordination systems show limited adaptability to rapidly evolving risks such as drone warfare and shifting sieges, with analysis hampered by data gaps and access constraints [316][310]. The UN Human Rights Office documented at least 880 civilian deaths from drone strikes between January–April 2026, underscoring failures to enforce International Humanitarian Law and protect civilians and civilian objects [382][320].

UN and independent bodies consistently call for lifting bureaucratic impediments, stronger national institutionalisation of PSEA, enforcement of IHL, and scaled funding and support to local actors to restore access, protection, and policy implementation [303][331][304][318].

Demography

Demographic overview

According to the UN-backed Integrated Food Security Phase Classification (IPC), around 19.5 million people in Sudan—approximately 41% of the population—faced high levels of acute food insecurity through May 2026, including about 135,000 people in IPC Phase 5 (Catastrophe), more than 5 million in Phase 4 (Emergency), and roughly 14 million in Phase 3 (Crisis) [413][417][403]. The IPC also estimated that some 825,000 children under five will suffer from severe acute malnutrition in 2026 [413][403]. Related UN analyses further report severe service disruptions—about 40% of health facilities nonoperational—which compounds protection and survival risks for displaced and vulnerable populations [413][405].

Internal displacement remains large and dynamic: roughly 8.9 million people remained internally displaced in April 2026 while nearly 4 million people have returned to areas of origin, and IOM estimates that about 15 million Sudanese have been displaced internally or across borders since the conflict began [399][401][417]. Analyses note ethnic diversity (including Ingessana, Berta, and Hamaj) and competition over land and resources as drivers of localized intercommunal tensions where displacement concentrates populations [435][426]. The 2024 Sudan Smallholder Farmers Survey found farming household heads are overwhelmingly male (97%) and skew older—nearly two-thirds of household heads are aged 45 or older—with over half reporting some form of postprimary education [387].

Third country nationals

Multiple neighbouring and transit countries host sizable Sudanese populations and other nationalities, shaping humanitarian needs and access to services [394][383].

- Central African Republic: 36,414 Sudanese refugees hosted countrywide (23,324 in the Korsi settlement, Birao) with UNHCR coordinating registration, protection and multisector assistance as of 30 April 2026 [394][388][383].
- Chad: 926,421 registered arrivals from Sudan as of 26 April 2026, with women and children comprising 87 percent and acute SRH, WASH and protection gaps reported [390].
- Libya/Eastern North Africa: rights groups report over 240,000 Sudanese entering Libya since April 2023 and UNHCR had registered over 86,000 by November 2025, while IOM data show Sudanese made up a large share of Q12026 arrivals (36% of registered migrants), with widespread detention, abuse and reliance on smugglers [385][393].

- Egypt: large urban Sudanese populations face security campaigns, arrests and forcible deportations in early 2026, undermining access to services and prompting ad hoc returns at border points [384].

- Mixed movement routes and reception centres in Morocco and Western Africa register new arrivals from Sudan alongside Malian, Chadian, Guinean, Cameroonian, Eritrean, Ethiopian, Somali and Nigerian nationalities, concentrating service demand in major cities and creating geographic service gaps elsewhere [386][402].

These demographic mixes are increasing protection and service pressures: host countries report overwhelming SRH and GBV needs among predominantly female and child refugee populations, overcrowded and abusive detention or transit settings for Sudanese in Libya and Egypt, and concentrated demand that leaves border and rural reception areas underserved—complicating registration, protection monitoring and humanitarian delivery [390][385][393][386][402]. Additionally, targeted expulsions of South Sudanese and other non-Sudanese groups from within Sudan and across borders are producing complex mixed caseloads that strain reception capacity and heighten protection risks in transit and return sites [391][392].

Socio Cultural

Socio cultural overview

Sudan's crisis is shaped by deep social fragmentation and eroding community structures. Intellectual and civic decline—described in multiple analyses as a collapse of the civil sphere and a 'displacement' of collective awareness—is eroding shared national narratives and social cohesion, increasing fragmentation along centre-margin, regional and identity lines and complicating prospects for community reconciliation and survivor access to justice [491][444]. The emergence of parallel administrations and moves to formalise local control (e.g., RSF-aligned 'Tasis' structures, registration directives and proposed local policing) are politicising humanitarian space, straining neutrality, and deepening administrative and social fragmentation across communities [447][477].

Conflict-driven displacement has reshaped household composition and social networks, driven sharp asset and livestock losses, contracted cultivated land and increased reliance on non-agricultural labour and humanitarian assistance among smallholder farming households [484][468]. FAO, WFP and UNICEF warned that close to nine million people were internally displaced as of end-March 2026, and that widespread destruction of markets, health and water systems is isolating communities and disrupting traditional protection and justice pathways [460][455]. In Dar Zaghawa and other hard-to-reach areas, people are living dispersed or hidden in the bush with limited food, health and WASH services, relying on clan support or cross-border movement (notably into Chad) while remaining largely unreachable by humanitarian actors [455].

Deep stigma surrounds sexual violence in Sudan: survivors face ostracism, family rejection, diminished marriage prospects and victim-blaming, and cultural taboos—particularly in eastern Sudan—make disclosure and PSEA reporting extremely difficult, with some communities preventing women from attending awareness programmes [463]. Pre-existing gender inequalities and patriarchal household and community structures enable abuse, protect perpetrators from accountability, and reduce survivors' willingness and ability to report [463]. Areas under RSF control, other conflict-affected zones and remote rural locations have the highest SEA risk and lowest PSEA coverage, as insecurity and access constraints undermine prevention, outreach and reporting mechanisms [463].

Gender dynamics

Gender-based violence (GBV) remains widespread, systematic and severely underreported across Sudan in 2026, encompassing conflict-related sexual violence (including rape, gang rape, sexual exploitation, forced marriage and sexual slavery), intimate partner violence and abuse reported in displacement sites, host communities, transit routes, checkpoints and besieged or active conflict areas [459][458][479]. Female-headed households and adolescent girls are especially exposed to exploitation linked to extreme deprivation [459][458][479]. Women and girls face heightened protection risks while fleeing, collecting water or firewood, accessing services, or living in overcrowded sites, and long distances to fetch water or firewood expose them to harassment and sexual assault [459][496].

Strong stigma, victim-blaming, cultural taboos and fear of retaliation suppress reporting and discourage survivors from seeking justice or services [463][459]. Survivors often lack access to life-saving clinical care, psychosocial support and safe referral pathways—particularly in besieged or hard-to-reach areas where around 40 percent of health facilities are non-functional—further heightening risks of mortality and prolonged harm [459][461]. Humanitarian protection, sexual and reproductive health (SRH) and maternal health services critical for GBV survivors are severely constrained by funding shortfalls and access limitations, undermining case management and prevention capacity [493][461].

Cross-border displacement has produced large refugee flows highly skewed towards women and children, with 926,421 registered arrivals in Chad as of April 2026, of whom 87 percent are women and children [481]. Inadequate site infrastructure—including lack of lighting and non-functional water points—is increasing exposure to assault and exploitation [481]. Inter-agency PSEA and community engagement efforts have reached over 1.47 million people with messaging on sexual exploitation and abuse, and the response has engaged women-led organisations as trusted intermediaries to strengthen outreach and reporting pathways [463].

Impact of socio cultural drivers on aid

Patriarchal norms and cultural taboos create significant barriers to inclusive humanitarian programming. Women's participation in decision-making and programming is severely limited: the 2024 Smallholder Farmers Survey shows 97% of household heads are male, and survey implementation faced a severe gender imbalance with only 3.5% female respondents, reflecting barriers to women's access to phones and participation in assessments [484]. In eastern Sudan, cultural restrictions make it difficult to speak to women or invite them to awareness programmes, and religious or cultural interpretations are sometimes invoked to discourage reporting or to prioritise community reputation over survivors' rights [463][479]. Operational constraints—mobile network disruptions and low female phone ownership—further hinder effective communication, inclusion in consultations and reporting mechanisms [484].

Discrimination and stigmatisation based on ethnicity, tribal affiliation, perceived party affiliation, displacement status, gender, disability and lack of documentation restrict access to assistance, services and safe movement, reinforcing tensions between host communities and displaced populations [479][458][459]. Shifts in allegiance among native administration leaders affect local power dynamics and can influence humanitarian access negotiations at community level [513][471].

Local civil society organisations, mutual aid groups and trusted intermediaries are critical to reaching affected communities and delivering survivor-centred approaches [463][471]. However, areas under RSF control and remote conflict zones face the lowest PSEA coverage and most severe access constraints, leaving the most at-risk populations with the least support [463][471].

Environment

Environmental conditions

Sudan's displacement crisis is heavily shaped by flood risk, water scarcity, and climate variability. An April 2026 IMPACT flood susceptibility analysis estimates about 6.91 million IDPs (~1.39 million households) are exposed to medium-to-very-high flood susceptibility across 183 localities, with over 4 million IDPs (~818,900 households) exposed to high or very high flood susceptibility across 163 localities covering approximately 1,195 mapped IDP sites [557]. The analytical framework underpinning this assessment was conducted in 2024, updated with 2026 population figures, and is considered valid for five years [557]. Priority state-level IDP exposures to high/very-high flood susceptibility include:

- North Darfur: ~1.10 million [557]
- South Darfur: ~1.01 million [557]
- Central Darfur: ~0.82 million [557]
- East Darfur: ~0.70 million [557]
- Gedaref: ~0.64 million [557]
- Northern: ~0.49 million [557]
- River Nile: ~0.46 million [557]
- White Nile: ~0.41 million [557]
- Blue Nile: ~0.36 million [557]

Localities with particularly pronounced exposure include Madeinat Al Gedaref, Beliel, Tawila, Wasat Jabal Marrah, Ed Damazine, Ad Du'ayn, Aj Jabalain, Al Fasher, Merwoe, Barbar, Kebkabiya, Ad Dabbah and Abu Hamad [557]. Seasonal rainy-season flood risk is identified as a primary preparedness priority, with recommended anticipatory actions including pre-positioning, site maintenance, disaster risk reduction (DRR), and flood-sensitive shelter and site-design measures ahead of peak rains [557]. Regional climate variability—marked by extreme rainfall and flooding across parts of Eastern Africa in early 2026—adds systemic risk to Sudan's conflict- and displacement-affected areas [548][553].

WASH deficits compound these environmental pressures at the national level: an estimated 17 million people lack access to safe drinking water and 24 million lack adequate sanitation [535]. Khartoum's water supply has effectively collapsed amid prolonged electricity outages and service disruptions, forcing households into long queues, rising prices, and declining water quality with growing concerns about waterborne diseases [587][589]. In Tawila, only 52% of IDPs had access to clean water—coverage halved in June and later fell to zero by September due to funding gaps—while 83% of households lacked soap, 52% practiced open defecation, and 42% of latrines were unusable [583]. Fires in March (24 separate incidents driven by dry conditions, high winds, use of wood for shelter, and overcrowding) destroyed 1,571 shelters, 76 latrines, and associated WASH infrastructure, affecting over 7,855 people; SI intervened with water trucks, is rebuilding 44 latrines, and distributed hygiene kits to affected households [583]. SI's February 2026 assessments of returnee sites found water yards, boreholes, and handpumps destroyed; SI subsequently rehabilitated one pump, restored water yards, and distributed shelter, NFI, and hygiene kits benefiting thousands of returnees [583].

Environmental challenges

Sudan's environmental challenges are driven by the intersection of conflict, displacement, climate shocks, and infrastructure collapse. Drone and aerial attacks have damaged civilian infrastructure—including those reported on 8 May—reducing access to food, clean water, and health care and further disrupting essential water and health services [595]. Agricultural production faces compounding environmental constraints: water and irrigation challenges were the most significant production constraints in the 2023/24 winter season, contributing to reduced productivity in vegetables, fruits, and pulses, and compounding input access and price pressures for seeds, fertiliser, pesticides, and fuel for irrigation [531].

The Site Management Cluster warns that flooding amplifies protection and public health risks, with overcrowding and damaged sanitation increasing exposure to disease and gender-based violence [557]. Regional climate shocks—including abnormal rains and flooding—interact with conflict and displacement in Sudan, intensifying livelihood losses, environmental degradation, and the need for anticipatory risk financing and preparedness [548][553].

On the financing side, the Saudi Fund for Development signed a \$10 million MoU in January 2026 with Sudan and UNHCR to strengthen water supply systems in Sudan, including improvements to the Nile water system in Khartoum and solar-powered operations to enhance service resilience [593].

Agricultural calendar

Sudan's agricultural calendar is structured around two primary seasons. The winter (irrigated) season runs from October–December planting through a March harvest, supporting wheat, millet, maize, forage crops (e.g., alfalfa, fodder sorghum), and diverse horticulture; these crops are highly dependent on commercial inputs and fuel for irrigation, making them especially vulnerable to conflict-related economic disruptions [531]. Wheat is the dominant winter crop nationally, with Aj Jazirah alone accounting for roughly 60% of national wheat production [531]. The critical main rainy season planting window runs roughly June–October (with the lean season spanning June–September), and divergent seasonal outlooks with an elevated risk of below-average rains if El Niño strengthens would directly affect planting timing, yields, and harvest outcomes [525][535].

The Sudan 2024 Smallholder Farmers Survey (SSSP Working Paper), covering 2,101 farming households across 13 states, found that 28% of farming households were displaced in 2023 and 61% reported limited seed reserves or inability to purchase improved seed and fertiliser, undermining planting capacity for subsequent seasons [531]. Sudan imports approximately 54% of its fertiliser from Gulf countries, and the World Food Programme warns that Middle East-related supply disruptions have added shipping delays of up to approximately 25 days for deliveries bound for Sudan—threatening timely input availability ahead of the June–October planting season [532]. Rising fuel and fertiliser prices (fuel reported up approximately 30%) and local staple price spikes in Darfur markets in mid-May are already constraining farmers' input choices and irrigation capacity, likely prompting shifts in cropping decisions and reducing production [544][559].

Major irrigation and power infrastructure risks add further pressure: damage or disruption to Merowe Dam—which supplies an estimated 1,250 MW and supports irrigation for roughly 400,000 hectares—would have severe knock-on effects on irrigation systems, agricultural production, and livelihoods in Northern state and other dependent areas [569].

Environmental risks and mitigation

Heading into the 2026 rainy season, Sudan faces a severe shelter and NFI funding shortfall: only US\$35.3 million of a US\$179.4 million requirement has been secured (19.7% funded), leaving 15.4 million people identified in need of emergency shelter and NFIs while only 1.8 million are targeted and 1.6 million prioritized under current constraints [523][518]. The Site Management Cluster prioritizes early action, site-level DRR, shelter reinforcement, pre-positioning, and rapid shelter/NFI response within 24–72 hours, identifying priority localities in North, South, and Central Darfur, East Darfur, Gedaref, Northern, River Nile, White Nile, and Blue Nile (Priority 1), with a second tier including Aj Jazirah, Kassala, Red Sea, North and West Kordofan, and Sennar [557][523]. Cluster guidance requires a minimum rainy-season DRR package and protection-sensitive site design, and urges donors to provide rapid, flexible top-up funding, activate SHF envelopes, strengthen ICCG coordination, and prioritize DRR and site safety in partner plans and proposals [523].

A severe heatwave warning was issued for 16 May 2026 affecting parts of Southern Northern state, River Nile state, and western Red Sea state, increasing heat stress and health risks [522]. Looking ahead, seasonal forecasts for June–August 2026 are divergent, but the likely emergence of El Niño during that period could raise the chance of below-average rainfall and poor harvests later in 2026; regional analyses recommend scaling anticipatory disaster risk financing and index-insurance mechanisms for resilience in the Horn of Africa, including Sudan [525][548][534].

Conflict

Overview of the ongoing armed conflict

The war began in April 2023 between the Sudanese Armed Forces (SAF) and the Rapid Support Forces (RSF), which has reconstituted operational capacity through an alliance with the Sudan People's Liberation Movement North (SPLMN), enabling sustained multifront operations into 2025–26 [714][637]. A major escalation opened a new front in Blue Nile in late January 2026, with RSF/SPLMN and SAF offensives focused around Kurmuk, Baw and Geisan, with frequent shifts in territorial control driving rising displacement and infrastructure destruction [645][637]. Kordofan and Darfur remain active hotspots, with intense fighting reported around El Obeid, Dilling and Kadugli, and repeated artillery and drone strikes causing civilian casualties and damage to populated areas in May 2026 [645][723][726].

The UN Human Rights Office reported that armed drone strikes killed at least 880 civilians between January and April 2026 and accounted for more than 80% of recorded civilian conflict deaths in that period, warning the conflict is entering a "deadlier phase" as drones spread beyond previous frontlines [728][724]. Drone and explosive-weapon use, together with artillery shelling, have repeatedly struck markets, hospitals (forcing closures), Khartoum International Airport and displacement camps, severely damaging health infrastructure and causing healthworker casualties [728][705][722]. Reported external facilitation and the spread of armed drones, including allegations of crossborder logistics support and satellite imagery of camps near Ethiopia, point to increased weapon proliferation and the entrenchment of multiple armed groups across regions [637][727][728].

Humanitarian access is increasingly constrained by siegelike conditions, checkpoints, road blockages and bureaucratic impediments across Greater Darfur, Kordofan and other frontline areas, limiting deliveries and forcing negotiation with multiple local armed actors [706][631][637]. IPC and food-security analyses project Emergency (IPC Phase 4) outcomes expanding across Greater Darfur, Greater Kordofan and Blue Nile during the June–September 2026 lean season, with some localities at risk of Catastrophe (IPC Phase 5) if access and conflict dynamics do not improve [678][645].

Conflict stakeholders

The Sudanese Armed Forces (SAF) is the main state actor and depends heavily on allied armed groups—including BBMB, the Justice and Equality Movement (JEM) and the Joint Forces (Mushtaraka)—for manpower, funding and operational reach. However, recent tensions with some allied factions have weakened SAF's alliance structure and raised the risk of localised confrontations in Khartoum, where allied groups are operating near markets and commercial areas [637]. The Rapid Support Forces (RSF) is the primary nonstate

adversary; it launched an early 2026 offensive in Baw county, Blue Nile state to secure routes toward central Sudan and the Roseires Dam, and has reconstituted multifront capacity through an alliance with Abdelaziz Adam al Hilu's SPLMN (AI Hilu) [637].

Both sides draw on regional and transnational ties: SAF has accused South Sudan and Ethiopia of enabling RSF mobilisation, and satellite imagery in early 2026 showed a logistics camp near the Ethiopian border, while US designations have alleged some armed groups received training or support from Iran, and reporting has linked UAE-connected cargo flights to suspected arms transfers to the RSF [637][666][652]. Factional fragmentation is evident: high-profile defections from RSF to SAF and recurring internal disputes among militias indicate fragile alliances, while RSF and allied militias have consolidated territorial control in parts of Darfur, Kordofan and Blue Nile and are advancing parallel administrative arrangements in areas under their control [645][650][667].

The conflict's political and territorial objectives are intertwined: RSF advances aim to increase leverage over central Sudan, while SAF and its allies seek to preserve state control and political influence. External backers materially shape the capabilities and bargaining positions of both camps [637][655][652]. External diplomatic and political pressure is rising, with leaders such as France's president and AU/UN investigative missions publicly accusing outside backers of fuelling the war and calling for ceasefires, accountability and unhindered humanitarian access [655][665].

Impact of the conflict on the population

Conflict has driven massive population displacement and widespread loss of services. Humanitarian agencies report roughly 8.9–9.1 million people internally displaced and about 15 million Sudanese displaced internally or across borders since the war began in April 2023 [623][640][607]. Approximately 350,000 people were newly displaced between October 2025 and March 2026, driven by fighting in North Darfur, Greater Kordofan and Blue Nile [645], and more than 100,000 people fled to northern Blue Nile and neighbouring states since early 2026 [637][714][678]. Around 4.5 million people have crossed borders as refugees since the conflict escalated, adding regional pressure on host states [617]. About 4.1 million people have returned to areas of origin, most (~83%) from internal displacement, but returnees commonly face destroyed homes and collapsed services — in Khartoum, approximately 70% of returned homes are partially or totally destroyed [623][607].

- Food security and nutrition: an estimated 19.5 million people (~41% of the population) faced acute food insecurity through May 2026, including ~135,000 in IPC Phase 5 (Catastrophe), ~5 million in Phase 4 (Emergency) and ~14 million in Phase 3 (Crisis) [615].
- Child malnutrition is rising, with an estimated 825,000 children under five expected to suffer severe acute malnutrition in 2026 [631][615].
- Health and WASH systems are severely degraded: roughly 40% of health facilities are nonfunctional, an estimated 17 million people lack safe drinking water and 24 million lack adequate sanitation [615][611].
- Humanitarian access and response are critically constrained and underfunded: only approximately 20% of Sudan's 2026 Humanitarian Needs and Response Plan was funded as of April 2026, and attacks including drone strikes have damaged health facilities, aid convoys and IDP sites, causing civilian casualties and disrupting assistance delivery [611][621][645][617].

These factors combine to heighten protection risks, drive disease outbreaks and worsen mortality and malnutrition, particularly among displaced populations, women and children.

Peace negotiations

The UN Secretary-General's Personal Envoy for Sudan, Pekka Haavisto, concluded a Gulf visit (4–11 May) during which he urged immediate deescalation, protection of civilians, curbs on arms flows, support for a truce and sustained international coordination including the Quad and Quintet [639]. The United States reiterated its commitment to a political solution, emphasised support for ceasefire efforts, held the Berlin statement up as a platform to revive negotiations and warned it retained unspecified pressure options to push the parties toward talks [656]. Kenyan President William Ruto said Kenya is promoting a broader diplomatic framework that would bring Egypt, Saudi Arabia, the United States and the UAE together to revive peace efforts and pressure both military leaders back to negotiations [634].

The African Union and the United Nations reaffirmed intensified diplomatic engagement and called for accelerated AU-led mediation to secure a ceasefire and preserve Sudan's unity [671]. AU and UN missions warned of deepening atrocities in Darfur and Kordofan, demanded an immediate halt to attacks on civilians, unrestricted humanitarian access, credible ceasefire monitoring, full cooperation with the ICC and proposed an African-led accountability mechanism [665]. International meetings, including a recent Berlin conference that mobilised nearly €2 billion in pledges for Sudan, produced funding commitments but only limited diplomatic breakthroughs, leaving negotiations stalled on the ground [627].

Conflict outlook

The near-term trajectory points to intensification: ACAPS assesses a high risk of escalation, notably northward toward Ed Damazine and the Roseires corridor, and UN reporting warns that armed drones now drive the majority of civilian deaths and permit year-round strikes that sustain fighting through seasonal lulls [637][689][708]. RSF is positioned for a prolonged, multifront advance rather than a short tactical push, and control on the ground remains functionally fragmented among SAF, RSF, local tribal formations and informal de facto powers, enabling parallel administrations and further eroding central authority [644][637][691].

External factors — including allegations of cross-border facilitation from neighbouring states, increases in suspected UAE logistical and weapons flows to RSF, and broader Middle East tensions that raise the strategic sensitivity of the Red Sea/Port Sudan corridor — increase the likelihood of sustained and expanded RSF operations and wider regional spillover [637][666][635]. Prospects for a comprehensive negotiated settlement in the immediate term are limited given fragmented leadership, the absence of an all-inclusive negotiating framework and reduced international diplomatic momentum [644][691][689].

Humanitarian consequences are already severe and likely to worsen: ACAPS notes exposure of 50,000–100,000 people if fighting expands in Blue Nile, and sieges or intensified frontlines could drive Famine (IPC Phase 5) in parts of South Kordofan and North Darfur [637][645]. Neardaily strikes on civilian infrastructure continue to undermine humanitarian access and increase protection risks including genderbased violence, childprotection violations and psychosocial distress among newly displaced populations [708][734][637][735].

Displacement

Displacement overview

IDMC estimated Sudan hosted about 9.1 million internally displaced people at the end of 2025, the largest number of IDPs of any country and the third consecutive year Sudan topped the list [743][752]. Conflict-driven internal displacements reached a record 32.3 million globally in 2025, with Sudan remaining a principal concentration of protracted displacement [743]. Humanitarian and risk analyses in May 2026 indicate fighting continued to drive new and repeated movements into 2026, with ACAPS and food-security outlooks projecting further displacement during June–September 2026 across Greater Darfur, Greater Kordofan and Blue Nile [791][804].

Key hotspots for large-scale displacement or hosting include El Obeid and surrounding localities in North Kordofan, parts of South Kordofan (Dilling, Kadugli and the western Nuba Mountains), northwestern North Darfur (Um Baru, Kerno, At-Tine), Blue Nile (including Ed Damazine and arrivals from At Tadamon and Baw), Khartoum, and displacement hubs such as Kosti and Tawilla [788][804][748]. Recent movement examples include more than 100,000 people displaced since early 2026 from At Tadamon and Baw counties into northern Blue Nile and neighbouring areas, and about 2,250 people who fled Sheikan locality between 25 October and 30 December 2025 [788].

Returns have been reported in parts of the country but were often fragile and partial, while siege-like conditions, forced eviction, property destruction and restricted movement continue to prevent safe, sustained returns for many IDPs [746][784][800]. Humanitarian access constraints, overcrowded sites and strained services persist; shelter and site management planning prioritises flood-prone IDP concentrations in North, South and Central Darfur, Gedaref, Northern, River Nile, White Nile and Blue Nile for anticipatory and response action [740][803]

Living conditions of the displaced population

Displaced families are sheltering in informal settlements, schools and with host communities amid severe overcrowding. Site-level conditions are deteriorating — disease has been reported in camps (including bilharzia in Shaheeda Afandi), schools used as collective shelters face flooding risks, and localised shocks such as fires in Tawila destroyed 1,571 shelters and 76 latrines, affecting about 7,855 people [738][766][787].

The Shelter/ES-NFI cluster reported a critical funding shortfall for 2026 — US\$35.3 million secured of US\$179.4 million (19.7% coverage) — leaving 15.4 million people identified as needing shelter/NFI assistance but only 1.8 million targeted and 1.6 million prioritised. Analysis shows approximately 6.91 million IDPs are exposed to medium-to-very-high flood risk, and over 4 million are at high or very-high risk across 183 localities [750].

Access to basic services is severely constrained: an estimated 17 million people lack safe drinking water, 24 million lack adequate sanitation, and roughly 40% of health facilities are non-functional. Only 20% of Sudan's 2026 Humanitarian Needs and Response Plan had been funded as of April 2026, limiting reach — partners aimed to reach 4.8 million people per month between February and May but only about 3.13 million received assistance in February [751][753]. Nearly 19.5 million people face acute food insecurity, markets and agricultural assets have been severely damaged, and widespread displacement is reducing income opportunities and increasing aid dependency [751]. Repeated disease outbreaks (cholera, measles, malaria and others) and conflict-driven disruptions are accelerating malnutrition, with an estimated 825,000 children under five expected to suffer from severe acute malnutrition in 2026 [751][806].

Protection risks are acute: women and girls face heightened exposure to gender-based violence and unsafe childbirth, while discrimination, looting, forced evictions and destruction or occupation of civilian property are widespread. Persons with disabilities, older persons and female-headed households face particular exclusion from services [787][800][799]. Underfunding and access constraints are driving major assistance gaps and leave displaced populations highly exposed to worsening shelter, WASH, health, protection and food crises, especially ahead of the rainy season [750][751].

Population movements

IOM has documented large-scale, repeated displacement waves since April 2023, reporting roughly 805 displacement waves (approximately one new wave every 33 hours) and noting ongoing returns alongside continued displacement [736]. According to FEWS NET, approximately 8.9 million people remained internally displaced in Sudan in April 2026 after nearly 4 million returned, mainly to relatively calmer parts of eastern Sudan [803]. FEWS NET also estimated roughly 350,000 new displacements between October 2025 and March 2026, driven by fighting in North Darfur, Greater Kordofan and Blue Nile, and recorded some 28,020 people displaced in Blue Nile between 11 January and 2 April 2026 [803]. IOM further estimated 49,512 displaced people (9,899 families) in Blue Nile State between 11 January and 4 May 2026, representing a 77% increase from the previous reporting period [738].

A farming household survey provides additional detail on internal movement patterns: 35% of farming households reported at least one family member had left due to the conflict, 88% of those displaced household members remained within Sudan, and 57% of sampled farming households had hosted displaced family members since late 2023, with about 70% of hosting households receiving people from other states [739].

State- and route-level destination patterns from the same survey show predominantly internal moves within Khartoum (mainly to Omdurman) and high internal displacement in West and South Kordofan, contrasted with strong inter-state outflows from Sennar, Aj Zahirah and North Kordofan, while Kassala and Red Sea stand out for higher international out-migration [739].

Cross-border movements continue alongside internal displacement. Flow monitoring and IOM data show continued movement into Libya in early 2026, with Sudanese making up about 36% of registered migrants in the reporting period; routes have shifted via Egypt (Adi/Jugob) and Chad (Al-Kafra road arrivals doubled between January and March 2026) after official crossings closed [741]. Chad has taken in nearly 1 million Sudanese refugees, and UNHCR data show the Central African Republic hosted 36,414 arrivals from Sudan as of 30 April 2026 [789][761].

Drivers of displacement

Conflict is the primary driver of displacement: Sudan's civil war began on 15 April 2023 between the Sudanese Armed Forces and the Rapid Support Forces, and ongoing frontline shifts, attacks on civilians, looting, forced evictions, occupation and destruction of homes and property have driven repeated displacement and prevented safe return [752][743][800][799][811]. The conflict has regionalized, producing large refugee outflows that strain neighbours — Chad alone has absorbed nearly 1 million Sudanese refugees — raising risks of wider instability and secondary displacement [789][810].

Economic collapse — including currency depreciation, inflation and disrupted trade — together with severe disruption to agriculture (asset losses, reduced cultivated land and declining livestock) has eroded livelihoods, increased dependency on humanitarian assistance, and left fewer than one in four farming households food secure while over half are severely food insecure [739][768]. Humanitarian access constraints, siege-like conditions, bureaucratic impediments and border/corridor disruptions have limited aid delivery and contributed to fragile, often unsafe returns reported in 2025–2026 [768][756][743].

Integration

IOM reports about 4.1 million returnees across Sudan, with more than 80% returning from within Sudan to nine major states. Most returnees face widespread destruction of homes and near-absence of electricity, water, health and education services; in Khartoum, roughly 70% of homes to which people returned were partially or totally destroyed [748]. IOM identified the health, protection and education sectors as among the most affected by funding shortfalls, and prioritised improving services in areas of return, providing jobs and safe incomes for vulnerable families, and supporting integration with host communities to prevent renewed displacement [748]. Diaspora and local civil-society channels are also being used to support livelihoods and service delivery — IOM's Rapid Response Fund has channelled sub-grants to diaspora groups to deliver life-saving assistance and map diaspora humanitarian engagement [747].

Protection Cluster and risk analyses in May 2026 document pervasive discrimination, denial of services, theft, forced evictions and property destruction that hinder access to assistance and safe return, and warn that promoting returns without parallel rehabilitation of infrastructure and basic services would further strain humanitarian capacity [800][799][788]. Local authorities in Blue Nile State suspended emergency response committees in May 2026, disrupting assistance across multiple camps and host-community sites [738].

At the institutional level, a US\$10 million MoU signed in January 2026 between the Saudi Fund for Development, Sudan and UNHCR targets rehabilitation and expansion of water supply systems, including in Khartoum. Cluster guidance calls for coordinated site rehabilitation, community engagement, housing, land and property (HLP) due diligence, and multi-cluster action across the Shelter, WASH, Health and Protection sectors to enable safer local integration [808][740][744].

Humanitarian Access

Humanitarian access overview

Humanitarian access in Sudan is severely constrained by active hostilities, siegelike tactics, frequent attacks on supply routes and civilian infrastructure, and bureaucratic impediments that are preventing timely, largescale deliveries ahead of the rainy season [844][859][825]. Key indicators of the crisis include:

- Nearly 19.5 million people face IPC Phase 3 or above (Feb–May 2026) [825][844].
- About 135,000 people are in IPC Phase 5 across 14 hotspots in Darfur, South Darfur and South Kordofan at risk of famine [825][844].
- Close to 9 million people are internally displaced as of endMarch 2026 [825][827].
- The 2026 Humanitarian Needs and Response Plan was only ~19–20% funded by April 2026, leaving responses underresourced [859][825].
- Around 40% of health facilities are nonfunctional; ~17 million lack safe drinking water and ~24 million lack adequate sanitation, worsening access to services and aid effectiveness [825][844].

Humanitarian presence is concentrated in urban hubs and selective priority localities across Greater Darfur, Greater Kordofan, Blue Nile and Khartoum, while remote, besieged or datapoor localities (e.g., Um Baru, Kerno, At Tina, Kadugli, El Fasher, Dilling) remain largely inaccessible or underserved [825][844]. Delivery is led by UN agencies (WFP, FAO, UNICEF), INGOs, IOM, and a range of local civil society, womenled organisations and mutual aid groups, with repeated calls to channel more funding to local actors [825][859][815][832]. Modes of delivery include road convoys, crossborder and corridor operations — notably via Chad/Adré and routes such as AdDabbah into Darfur and White Nile into parts of South Kordofan — and air where security allows, but these routes are

regularly disrupted by attacks, border closures or corridor restrictions [819][832][825]. Legal and regulatory constraints, including bureaucratic impediments, visa delays, a dual/politicised governance environment and contested recognition dynamics, further restrict movement and operational presence for UN and NGO actors [844][824][819]. As a result, humanitarian actors are increasingly reliant on locally embedded organisations, community responders and mutual aid networks to reach besieged or hard-to-reach populations, while overall coverage remains well below identified needs [841][819][859].

Barriers to humanitarian access

Active hostilities — including drone strikes, repeated attacks on health facilities, and threats to aid personnel such as abduction and targeting of convoys and warehouses — have sharply increased security risks for humanitarian operations [817][841]. The Protection Cluster reports siegelike conditions, encirclement, checkpoints, road blockages and restricted movement that have denied or obstructed assistance in Greater Darfur, Kordofan and frontline localities including El Obeid [835][820]. Territorial fragmentation forces negotiation with multiple armed actors, further disrupting humanitarian supply chains and commercial flows [844][841][825]. Access constraints are most acute in areas flagged by IPC/FRC as highest risk — Um Baru, Kerno, At Tina and the Dar Zaghawa area in North Darfur, as well as parts of South Darfur, South Kordofan and Blue Nile (including Ed Damazine) — where siege conditions, displacement and inaccessibility prevent both assessments and lifesaving aid delivery [844][841].

Severe underfunding has compounded operational constraints: with the humanitarian response at only ~19–20% funded by April/May 2026, organisations have been forced to scale back services and withdraw support — MSF noted over 45 general healthcare centres in Central Darfur lost support — further reducing operational reach [860][825][817]. The Protection Cluster also documents discrimination and stigmatisation that exclude IDPs, returnees, female-headed households, persons with disabilities and undocumented people from distributions, health and other services, including at checkpoints [835][820].

Seasonal and environmental barriers further amplify these constraints: the approaching rainy season, flooded or impassable roads, and remote terrain — such as Jebel Marra areas reachable only by donkey and cut off during rains — necessitate prepositioning and disaster risk reduction measures [817][816][844]. Given the combined weight of these barriers, analysts note increasing reliance on local civil society, mutual aid groups and diaspora channels to reach populations that international responders cannot access directly [841][839].

Humanitarian access gaps

Humanitarian access gaps are most severe in areas under Rapid Support Forces control and across Greater Darfur, Greater Kordofan and parts of Blue Nile, with frontline localities such as El Obeid and Dar Zaghawa sites (At Tina, Um Baru, Kerno) reported as largely unreachable by responders [841][834][832]. The most underserved populations include internally displaced people, returnees and conflict-affected communities — particularly women and girls (including pregnant and adolescent girls), female-headed households, older people and persons with disabilities — who are being left without consistent assistance and protection services [835][834][871][832]. Critical shortfalls affect health (functioning clinics, medicines, maternal care), nutrition and WASH services, food access and protection/GBV response capacity, undermining lifesaving support in the worst-affected localities [871][844][835][832].

With the humanitarian response at around 19% funded four months into 2026, the scale, timeliness and geographic coverage of aid deliveries remain critically limited [860]. Responders increasingly rely on local civil society and mutual aid networks to negotiate access, and limited alternative modalities such as mobile clinics have been used to reach some hard-to-reach populations, but these measures remain insufficient to close major coverage gaps [841][818].

Information & Communication

Information landscape

The operational information environment in Sudan is highly constrained by insecurity and parallel authorities. The RSF-affiliated National Humanitarian Access Authority (TASIS) extended a registration deadline to June 13, reporting compliance by 36 of 43 international and about 138 national organisations as of mid-May 2026 [945], while independent reporting indicates most foreign organisations have not engaged the process (sources suggested only ~2% initiated it), with some agencies preparing staff withdrawals that disrupt operations and data flows [946]. ACAPS released a Sudan risk analysis on 13 May 2026 relying on secondary data review and a limited number of key-informant interviews with INGO/NGO responders, reflecting the broader dependence on secondary sources and constrained primary verification in access-restricted areas [898]. Cluster information systems (3W/4W tools and dashboards) are in use but face challenges securing timely, consistent partner data, limiting monitoring reliability and the timely use of data for decision-making [921]. Risk assessments further identify uneven data availability and limited coverage in areas under RSF control, Greater Darfur, Greater Kordofan and parts of Blue Nile, producing under-identification and underestimation of needs in hard-to-reach locations [918][923].

Key operational and needs indicators reported in May 2026 include:

- About 9 million people remain internally displaced, 4.5 million have crossed borders, and nearly 4 million people have returned to home areas [919].
- Nearly 350 drone attacks were recorded in Sudan since the start of 2026, with drone strikes reportedly killing close to 700 civilians (including 245 children) in the first three months of 2026, disrupting humanitarian operations and data collection [942][919].
- FAO, WFP and UNICEF warned that nearly 19.5 million people face acute food insecurity [936].
- UN briefings noted humanitarian deliveries in Kordofan in May 2026, including WASH reaching approximately 85,000 people in Sheikan and Ar Rahad, blankets and mosquito nets distributed to 2,000 displaced families, and cash assistance to 200 families in April, illustrating selective operational reporting from locations with access [944].

- UNHCR mapping (CAR, 30 April 2026) documents cross-border operational presence and refugee figures linked to the Sudan situation (e.g., 23,324 refugees in Birao/Korsi), showing reliance on 3W-style operational datasets for cross-border response planning [924].

Protection actors and surveys report pervasive underreporting—particularly of gender-based violence—due to stigma, fear of retaliation and collapsed services. The 2024 Multi-Sector Needs Assessment found 52% of respondents unaware of complaint and feedback mechanisms and 40% not told that aid is free, indicating major gaps in information accessibility and community awareness [929][876]. Overall, humanitarian information in May 2026 is produced through a mix of cluster tools, agency briefings, secondary analyses and limited key-informant interviews, but remains fragmented, access-dependent and affected by underreporting and parallel administrative constraints that reduce data reliability and hinder information sharing [898][921][929][945].

Barriers to information

A key structural barrier is that many PSEA and awareness materials are produced only in English and Arabic, while communities speak dozens of local languages; translation to local languages remains incomplete and child- and disability-accessible formats are limited [876]. Complaint and feedback mechanisms are fragmented, poorly coordinated and unevenly implemented across clusters, weakening two-way information flows and accountability to affected communities [921]. The 2024 Multi-Sector Needs Assessment found 52% of respondents unaware of these mechanisms and 40% not told that aid is free, though this finding is most relevant to the information landscape context above.

Cellular network disruptions—especially in Darfur—severely limit phone-based access: the Sudan 2024 Smallholder Farmers Survey recorded 5,613 failed SIM/activation attempts in Blue Nile, and only 3.5% of survey respondents were women, reflecting gendered gaps in mobile ownership that limit women's participation in phone-based information channels [927]. About one-third of farming households reported owning neither a radio nor a television, limiting the reach of media-based outreach [927]. Stigma, fear of retaliation and the collapse of services suppress reporting of gender-based violence, obscuring survivors' access to information and available reporting channels [911]. Community radio and SMS/hotline approaches remain under-utilised but viable channels; existing local broadcast infrastructure could partially mitigate literacy and mobile-access gaps if scaled and adequately funded [876].

Communication channels

Humanitarian actors and affected communities primarily exchange information through community FM radio, mobile phone-based platforms (hotlines, SMS and two-way reporting), social media, community dialogues, public theatre and community clubs [876]. Inter-agency toll-free hotlines and Community Feedback Mechanisms (CFMs) have been scaled up to enable two-way engagement and anonymous reporting, with inter-agency PSEA outreach reported to have reached over 1.47 million people with messaging on sexual exploitation and abuse and available reporting channels through combined media and community approaches [876]. Women-led and community organisations serve as critical face-to-face intermediaries with deeper local trust and have been purposively engaged in PSEA outreach and reporting pathways [876].

Community radio infrastructure can provide broad, linguistically inclusive reach—for example, community stations are present in five of seven localities in River Nile—supporting outreach to low-literacy and hard-to-reach groups [876]. Mobile networks and phone-based tools are effective enablers for discreet reporting and real-time feedback but are undermined by intermittent connectivity and large-scale cellular disruptions, which have left regions—particularly Darfur—unreachable by telephone and limited women's participation due to lower mobile ownership [927][876]. Prolonged power outages and fuel shortages further disrupt telecommunications, affecting long-range communications and network functionality and constraining real-time alerts and digital information access [918][902].

Health clusters and WHO produce epidemiological bulletins and cholera dashboards that guide response, though information products are often narrow in focus and keeping data updated across rapidly changing areas remains a persistent challenge [921]. Preparations for the 2026 rainy season include strengthening surveillance and early-warning using phone and radio alert systems and community-based surveillance for cholera preparedness [877]. Information and data collection are severely constrained by insecurity, access restrictions and uneven reporting in conflict-affected areas, producing delays, coverage gaps and weak feedback loops for corrective action [918][898]. Persistent funding shortfalls and severe humanitarian access constraints—including siege-like tactics, bureaucratic impediments, attacks on supply routes and destruction of infrastructure—limit the scale, reliability and timeliness of information systems and two-way communication across Sudan [919][935][956].

Misinformation and rumors

Rumours about foreign or military activity—for example, persistent claims of an Egyptian presence near Pagak and along Nile-border corridors—circulate widely in the region and carry geopolitical weight despite no publicly verified evidence, shaping public perception and diplomacy [937]. Data gaps, uneven reporting and communication outages, particularly in areas under RSF control, Greater Darfur, Greater Kordofan and parts of Blue Nile, produce information vacuums that increase uncertainty and the risk of misinformation spreading [918]. The ICRC reported that missing-persons cases in Sudan have surpassed 11,000—a rise of more than 40% over the past year—and documented families being targeted with false leads and exploitative misinformation, while facilitating over 80,000 phone calls to reconnect separated family members during the first three months of 2026 [899].

Humanitarian assessments and guidance recommend community-based reporting channels through trusted intermediaries such as women-led groups, religious leaders and community committees, alongside mobile hotlines, SMS and community FM radio, to provide discreet two-way communication, counter rumours and link reporting to survivor-centred services; however, field-level operationalisation remains uneven [876].

Preferred sources of information

Community-based FM radio in local dialects and mobile phone-based platforms (SMS, real-time hotlines, two-way communication and anonymous digital reporting) are identified as preferred channels to reach low-literacy and hard-to-reach populations [876]. Recommended trusted intermediaries include trained female mobile outreach staff, community-based facilitators, women-led organisations, religious leaders where appropriate, and community committees (WASH, health, Food Security and Livelihoods) to enable confidential reporting and survivor-centred referral pathways [876]. Satellite and space-based internet combined with solar power is increasingly relied upon to maintain connectivity where local networks and power are disrupted [931]. Diaspora networks and humanitarian actors also use centralised SMS and WhatsApp alert platforms and web portals for crisis communication and information dissemination [885].

In displacement settings, organisations facilitate connectivity directly, including:

- 71,000 phone calls facilitated to connect families [909]
- 57,084 beneficiaries provided with internet access for family communication [909]
- 18,628 devices charged to enable communication [909]

Health & Nutrition

Damage to health infrastructure

Humanitarian assessments indicate roughly 40% of health facilities across Sudan are non-functional, substantially limiting access to care [971][969]. The UN Human Rights Office warned on 11 May 2026 that armed drone strikes killed at least 880 civilians between January and April 2026 and that health facilities were hit at least 12 times during that period, forcing some hospitals and clinics to close or severely limit operations [1011][993]. WHO reported nearly 100 people were killed in strikes on multiple health facilities between January and April 2026, including an attack on Al-Daein Teaching Hospital on 20 March that killed more than 60 people, an attack on Al Jabalain Hospital on 2 April, and a drone strike that destroyed a State Ministry of Health warehouse on 4 April, disrupting drug supplies bound for South Kordofan [969]. MSF warned of a surge in drone strikes in 2026, citing INSO data recording nearly 350 drone attacks since the start of the year, with 11 incidents reported to have struck health facilities [997]. The Protection Cluster has identified attacks on hospitals and other health infrastructure as a central and escalating pattern of protection violations nationwide in 2026 [1012].

Attacks on health centres

Insecurity Insight recorded over 745 attacks on health care since April 2023, with 206 health workers killed, 15 kidnapped, 126 arrested, health facilities damaged more than 200 times, and 55 incidents where drones affected health care [976]. Notable incidents in 2026 include the strike on Al-Daein Teaching Hospital on 20 March that killed more than 60 people, a drone attack damaging Al Jabalain Hospital in White Nile State on 2 April, and a 4 April drone strike that destroyed a State Ministry of Health warehouse in White Nile, disrupting drug supplies bound for South Kordofan; a separate March strike on Ed Duweim hospital reportedly also killed at least 60 people [969][1020]. Routine immunization coverage dropped from 94% in 2022 to 48% in 2025 as a direct consequence of attacks on and closures of health facilities [969].

Availability and cost of medicines and health nutrition services

Medical and nutrition supply chains face severe strain across multiple fronts. The 4 April 2026 drone strike that destroyed a State Ministry of Health drug warehouse in White Nile caused local stockouts and disrupted deliveries to states such as South Kordofan [969][1046][997]. A surge in drone strikes and other attacks—with INSO and MSF recording hundreds of strikes since early 2026—combined with rising fuel and shipping costs, threatens deliveries through Port Sudan, the main entry point for medical and humanitarian supplies [997][1003][975]. Chronic underfunding compounds the crisis: only about 20% of Sudan's 2026 Humanitarian Needs and Response Plan was funded by April 2026, limiting procurement, prepositioning and distribution of essential medicines and nutrition commodities at national and subnational levels [971][967][1016]. Nearly 100,000 SAM admissions occurred between January–March 2026, overwhelming treatment capacity and demand for therapeutic feeds and related medicines against a projected caseload of 825,000 children under five with SAM in 2026 [971][967][977]. Field reports describe frontline health centres in areas such as eastern Jebel Marra operating without medicines, staff or basic supplies, forcing patients to travel long distances and indicating acute shortages at facility level [1038].

Impact of the conflict on doctors and health staff

Violence has directly and systematically targeted health workers. The Sudan Medical Network and Preparatory Committee of the Sudan Medical Association documented 511 health workers injured since the conflict began and 84 medical personnel remaining in detention—including 20 in El Fasher and 64 in Nyala—with additional cases of enforced disappearance and arrest of doctors [999]. Insecurity Insight recorded 206 health workers killed, 15 kidnapped, and 126 arrested across over 745 attacks on health care since April 2023 [976]. Humanitarian medical organisations report staff fear working and face acute operational constraints: MSF warned of drone strikes occurring dangerously close to aid teams (reported within 100 metres), noted staff reluctance to attend facilities, and highlighted funding losses and centre closures that exacerbate staff shortages and reduce services [997][1007][979]. The cumulative effect is severe service collapse, with roughly 40% of health facilities non-functional, sharply increasing pressure, risk and workload for remaining doctors and health staff while limiting civilian access to care [969][967].

Disease outbreaks

Sudan is experiencing widespread outbreaks of measles, malaria, dengue, polio, hepatitis E, meningitis, diphtheria, mpox (monkeypox) and recurrent diarrhoeal diseases, while cholera was declared contained in January 2026 [972][971][981][1053]. Disrupted immunization has left about 1.5 million children under five unprotected, prompting emergency vaccination campaigns; a polio campaign launched on 28 April targeted over 4.3 million children across seven states [972][969]. Civilian authorities in Jebel Marra reported rising monkeypox cases on 15 May 2026 and called for large-scale vaccination campaigns and delivery of medicines and treatment support [1053]. Despite the cholera containment declaration, partners warn of high risk of recurrence with the approaching rainy season and persistent WASH gaps; Solidarités International reports only 52% of IDPs in Tawila have access to clean water and widespread latrine and hygiene shortfalls, prompting targeted cholera preparedness and CATI plans [972][991][1014]. Repeated disease outbreaks are accelerating nutritional deterioration, with an estimated 825,000 children under five expected to suffer from severe acute malnutrition in 2026, stretching therapeutic and inpatient capacity [971][981]. FAO, WFP, UNICEF, WHO and other health partners have called for immediate scale-up of funding, protection of health workers, strengthened facility- and community-based surveillance, and rapid vaccination and WASH interventions, noting that only 20% of Sudan's 2026 HRP was funded as of April and humanitarian reach remains below planned monthly targets for February–May 2026 [971].

Health and nutrition needs and priorities

As of mid-May 2026, health and nutrition needs in Sudan are acute and deteriorating:

- Nearly 19.5 million people face high levels of acute food insecurity (IPC Phase 3+) [967][975].
- About 135,000 people were classified in IPC Phase 5 (Catastrophe) [967][977].
- An estimated 825,000 children under five are expected to suffer Severe Acute Malnutrition (SAM) in 2026 [967][981].
- More than 8.9 million people are internally displaced [981][972].

Humanitarian response coverage remains far below needs: partners aimed to reach 4.8 million people per month but reached an estimated approximately 3.1 million in February, and the 2026 Humanitarian Needs and Response Plan was only about 20% funded by April 2026, limiting scale-up and access to treatment [972][967][969]. Access, funding shortfalls, and information gaps continue to limit response planning and service provision in the most affected and hard-to-reach localities [981][972][967]. Priority operational needs include rapid scale-up of SAM treatment (surge staffing, supplies, expansion of community-based management of acute malnutrition), restoration and protection of health services and vaccination campaigns, strengthened WASH to reduce disease transmission, and urgent removal of access and bureaucratic constraints to enable sustained humanitarian delivery [975][967][1002].

Education

Impact of the crisis on school infrastructure and personnel

Education services across Sudan are widely disrupted: many universities and examination centres have been closed or rendered inaccessible, and the alternative Sudanese secondary-school certificate examinations planned for eastern Chad on 11 May 2026 were postponed due to lack of facilities and coordination [1106][1109]. Schools and students have been directly affected by violence, with rising drone strikes and other attacks increasingly impacting civilian infrastructure including displacement camps and schools; a 11 March drone strike killed 17 girls, intensifying safety and protection concerns for schools and exam takers [1115][1106]. Returned areas and places of origin often lack the basic services needed for schooling — around 70% of homes to which people returned in Khartoum were partially or totally destroyed, and areas of return face a near-absence of electricity, water, health, and education services, undermining school functionality [1076]. Protection monitoring documents grave violations affecting education, including attacks on schools, recruitment of children by armed actors, family separation, and large numbers of unaccompanied and separated children, raising demand for education-related protection, psychosocial support, and safe learning environments [1069].

Access to education

UNICEF estimates that more than 19 million children in Sudan are out of school as of 2026, with roughly 70% of girls among them [1104][1108]. Widespread school closures, attacks on education infrastructure, and the occupation of school buildings for displacement have removed learning spaces and interrupted examinations and higher education, forcing some students to travel long and dangerous distances — with reported journeys of three to four kilometres to reach remaining examination centres [1101][1106][1076][1069]. Siege-like conditions, checkpoints, road blockages, and severely restricted humanitarian access are preventing enrolment and regular attendance, and are driving harmful coping mechanisms — including increased early and forced marriage — that disproportionately affect girls and raise dropout risks [1069][1091][1104][1108]. Continued attacks on schools, recruitment of children by armed actors, and explosive ordnance contamination further deter attendance and endanger learners across conflict-affected areas [1069][1091]. Access is further constrained by mass displacement, discriminatory exclusion at checkpoints and in assistance distribution, and severe funding shortfalls that have left the education sector among the most affected sectors in humanitarian response planning [1091][1076].

Education needs and priorities

Key needs include the rehabilitation of damaged and non-functional school buildings, establishment of temporary learning spaces, restoration of WASH services in schools, provision of trained teachers and learning materials, and targeted psychosocial support for children affected by the conflict [1066][1091][1076][1069]. Severe gaps persist: widespread destruction of education infrastructure, access constraints, and acute funding shortfalls have left the education sector critically constrained and among the most affected sectors in humanitarian responses [1076][1094][1066]. Humanitarian actors — notably UNICEF, IOM, and UNHCR partners — are conducting child protection, family reunification, and service restoration activities, and are calling for urgent scaling up of funding and safe, sustained humanitarian access to restore basic services including education, as underscored in May 2026 appeals [1068][1076][1080][1075]. Coordination mechanisms among UN agencies, international and local NGOs, and government actors are in place, but ongoing insecurity, access restrictions, and underfunding continue to impede the delivery of emergency schooling and durable education-in-emergencies interventions as of May 2026 [1099][1076][1075].

WASH

Damage to water infrastructure

ACAPS warned on 13 May 2026 that damage to the Merowe Dam or its associated electricity grid could disrupt WASH systems and water pumping, causing prolonged outages and reduced water services across northern, central, and eastern Sudan [1120]. The Khartoum State Government noted that several water stations have had to run on gasoline generators — the Al-Manara station alone requiring approximately 80 barrels of gasoline per day — compounding supply constraints amid fuel shortages [1119]. Drone strikes have struck critical water points, including reports that the El Tina/Andour well in North Darfur was rendered inoperable, triggering fears of severe water shortages and higher disease risk for nearby communities [1127][1124].

In Tawila, fires and conflict-related damage destroyed 76 latrines, damaged water storage infrastructure (including a water bladder), and affected WASH facilities serving over 7,855 people, prompting emergency water trucking, latrine reconstruction, and other rapid WASH measures in March–April 2026 [1133].

Access to clean water

An acute WASH crisis persists across Sudan, with conflict, infrastructure damage, and power outages severely disrupting water supply and increasing disease risk [1139]. FAO, WFP, and UNICEF warned on 15 May 2026 that an estimated 17 million people lack access to safe drinking water and 24 million lack adequate sanitation [1122]. The IPC analysis covering February–May 2026 assessed 195 localities and flagged limited humanitarian access and damaged water systems as worsening WASH conditions and disease risk [1153]. ACAPS (13 May 2026) warned that prolonged electricity outages threaten water-pumping services and cited the 2024–2026 cholera outbreak — with approximately 124,400 suspected cases and around 3,600 deaths — as evidence of outbreak risks when WASH systems fail [1120]. Attacks have directly damaged water infrastructure, including reported destruction of the only well in El Tina (North Darfur), producing acute shortages and reliance on unsafe sources [1132]. In Tawila, only 52% of IDPs have access to clean water, with coverage reportedly halved at one point and falling to zero by September in the period described; local assessments further reveal that 83% of households lack soap, 52% practice open defecation, and 42% of latrines are unusable [1133].

Contamination of water sources

The Sudan Site Management and Shelter/NFI Preparedness and Response Plan (May 2026) prioritizes anticipatory action for flood-prone displacement sites, calling for early disaster risk reduction (DRR) measures to address drainage, site safety, and WASH disruptions ahead of the 2026 rainy season [1178][1174]. The ES/NFI Cluster warns that flooding produces waterlogged shelters, stagnant water, damaged WASH facilities, and increased waste-related vector risks, and that shelter inputs alone cannot resolve drainage, waste, or vector control needs [1151]. Solidarités International (SI), with support from the Swiss Agency for Development and Cooperation, is implementing targeted cholera WASH interventions in Darfur — including rapid case-area targeted interventions (CATI), household and latrine disinfection, emergency water chlorination, and distribution of cholera kits and aquatabs — reaching 20,471 people in West Darfur, planning to reach over 15,000 in Central Darfur [1133]. Although cholera was reported as contained since January 2026, the fragility of WASH services keeps transmission risk high as the rains approach [1131][1133]. These combined site-management, WASH rehabilitation, and community hygiene promotion actions are being advanced ahead of the rains to reduce water contamination and the risk of water- and vector-borne disease transmission in high-risk IDP sites and host communities [1178][1151][1133].

Water needs and priorities

Humanitarian WASH interventions — documented mainly by Solidarités International (SI) — span emergency water delivery, sanitation repairs and construction, hygiene distributions, and behaviour-change activities. SI reported delivering 55 m³/day to Feina since 1 April 2026 (reaching approximately 7,000 people), constructing 30 two-door latrine blocks for 3,000 people, rebuilding 44 destroyed latrines restoring sanitation for 2,200 people, distributing hygiene kits to 1,042 households, replacing a damaged water bladder, and drilling two boreholes in North Darfur to benefit over 10,000 people [1133]. SI also reached 20,471 people in West Darfur with hygiene promotion and waterpoint rehabilitation, and plans include household and latrine disinfection, emergency chlorination, CATI, cholera-kit distributions, and door-to-door hygiene promotion [1133]. Despite these efforts, response effectiveness remains localized. Sudan's 2026 Humanitarian Response Plan was only approximately 20% funded as of April 2026, severely constraining sustained WASH programming [1122][1123][1133]. Major challenges include security-driven access constraints, damage to water infrastructure, power outages disrupting pumping and treatment, and operational complications ahead of and during the rainy season [1133][1120][1132][1119]. Humanitarian actors call for immediate, sustained funding and prioritized preventive WASH measures — including chlorination, waterpoint rehabilitation, hygiene kit and aquatab distributions, CATI, and hygiene promotion — to reduce disease risk and protect vulnerable displaced and returnee populations as rainy-season threats intensify [1133][1122][1123].

Sanitation & Hygiene

Access to adequate sanitation facilities

According to a joint FAO–WFP–UNICEF news release (15 May 2026), an estimated 24 million people in Sudan lack access to adequate sanitation facilities, while 17 million lack access to safe drinking water [1199][1202][1216]. In Tawila (North Darfur), Solidarités International reported in May 2026 that only 52% of IDPs had access to clean water, 52% of households practised open defecation, and 42% of latrines were unusable [1194]. Fires in March 2026 destroyed 1,571 shelters and 76 latrines, affecting over 7,855 people and further degrading sanitation infrastructure [1194]. In South Darfur's fast-growing Feina IDP site, there were no functional sanitation facilities and widespread open defecation as of March 2026 [1194]. In response, SI implemented emergency WASH interventions including rebuilding 44 destroyed latrines (restoring sanitation for approximately 2,200 people), constructing 30 two-door latrine blocks benefiting around 3,000 people in Feina, and distributing hygiene kits to 1,042 households [1194].

Prevalence of WASH coping mechanisms

In Tawila IDP sites, SI reported severely deteriorating WASH conditions, with water coverage halved in June 2025 due to funding gaps and falling to zero by September 2025 [1194]. Key sanitation and hygiene coping indicators reported include:

- 83% of households lacking access to soap [1194]
- 52% of households practising open defecation [1194]
- 42% of latrines unusable in assessed sites [1194]

- 76 latrines destroyed as part of 1,571 shelters lost in March 2026 fires, affecting over 7,855 people [1194]

Humanitarian partners reported deteriorating conditions across displacement sites—overcrowding, poor sanitation, and the adoption of negative coping mechanisms—heightening disease and protection risks ahead of the rainy season [1223]. At national scale, an estimated 17 million people lack access to safe drinking water and 24 million lack adequate sanitation, exacerbating reliance on harmful WASH coping strategies [1199].

Availability and cost of hygiene items

Humanitarian actors report severe constraints in both the availability and affordability of hygiene items across Sudan, driven by damaged WASH infrastructure, funding gaps, and market disruptions [1199][1194]. In Tawila, 83% of households lacked access to soap [1194]. Market and transport shocks are increasing costs: WASH service disruptions and parallel-market dynamics have pushed up the price of water in Khartoum—for example, water purchased for 5,000 pounds was resold at 20,000 pounds—exacerbating affordability problems for hygiene-related needs [1215]. Qatar Charity distributed hygiene assistance in Khartoum/Omdurman on 11 May 2026, delivering 500 personal hygiene kits alongside 470 food baskets, 1,000 blankets, and medicines to around 400 displaced families [1264][1265]. SI's ongoing cholera-focused WASH responses include household and latrine disinfection, emergency chlorination, and cholera-kit and hygiene-kit distributions, reporting reach of 20,471 people in West Darfur and plans to reach over 15,000 in Central Darfur (including aquatab distributions), while drilling two boreholes in North Darfur to serve over 10,000 beneficiaries [1194].

WASH driven disease risks

Humanitarian assessments warn that widespread WASH failures amid conflict, displacement, and damaged infrastructure are driving heightened risks of cholera, diarrhoeal diseases, measles, and other outbreaks—risks expected to intensify during the rainy season (June–September) [1199][1196][1214]. A prolonged cholera crisis illustrates the scale of the threat: the outbreak declared in August 2024 lasted until March 2026, with more than 124,400 suspected cases and around 3,600 deaths [1209]. Darfur saw over 12,000 cholera cases between May–September 2025, including 7,435 cases in Tawila locality during that period [1194]. Damage to power and fuel infrastructure—such as attacks on the Merowe dam and power grid—reduces pumping and treatment capacity, undermining WASH systems and contributing to waterborne disease spread and prolonged outbreaks [1209]. Overcrowding in displacement sites and non-functional WASH facilities are linked to outbreaks of measles and other vaccine-preventable or waterborne diseases and constrain vaccination and treatment efforts [1261][1199][1214]. Humanitarian actors are implementing targeted WASH responses and cholera preparedness: SI reported providing 55 m³/day of water (reaching approximately 7,000 people), constructing 30 two-door latrine blocks (serving approximately 3,000 people), decommissioning non-functional latrines, and conducting chlorination and hygiene distributions in hotspot areas [1194].

Sanitation and hygiene needs and priorities

Médecins Sans Frontières described conditions in Tawila and other Darfur sites where emergency latrines are already full, long queues and rapidly diminishing water supplies are forcing people to defecate outdoors, and waterborne disease risk is rising ahead of the rainy season [1204]. Site management and Shelter/NFI cluster guidance emphasizes the need for anticipatory, protection-sensitive WASH and site-safety actions—including disaster risk reduction, latrine repair, safe water pre-positioning, and sanitation reinforcement—before peak rains to reduce disease and protection risks in overcrowded, flood-prone displacement sites [1248][1239]. SI is scaling cholera preparedness WASH measures through rapid CATI deployments, household and latrine disinfection, emergency water chlorination, and distribution of cholera kits and aquatabs, having reached 20,471 people in West Darfur and over 15,000 in Central Darfur through waterpoint rehabilitation, chlorinated water distributions, and hygiene promotion [1194]. Funding gaps remain a critical constraint, having already caused water coverage in Tawila to halve in June 2025 and fall to zero by September 2025, underscoring the urgency of sustained financing for WASH operations ahead of the next rainy season [1194][1197].

Food Security

Availability and cost of food

FAO, WFP and UNICEF warned on 15 May 2026 that nearly 19.5 million people in Sudan face acute food insecurity (IPC Phase 3+), with around 135,000 people in IPC Phase 5 (Catastrophe), more than 5 million in Phase 4, and 14 million in Phase 3 [1300]. The agencies estimate that about 825,000 children under five will suffer from severe acute malnutrition in 2026 [1300]. FAO's CFSAM/FEWS NET reporting estimated national cereal production for 2025/26 at 5.2 million tonnes — 22% below last year — leaving an estimated 2.8 million tonne deficit that reduces market supply and national availability [1292]. Conflict-driven constraints — including siegelike conditions, trade bans on goods into RSF-controlled areas, attacks on markets and supply routes, and corridor/border disruptions — have restricted commercial flows, pushed reliance onto longer informal routes, and tightened local market stocks and transport capacity [1301][1303][1314]. Market prices remain extremely elevated: average sorghum, millet and wheat prices in March 2026 were 270–430% higher than March 2023, and mid-May reporting documented sharp local price spikes and shortages in areas such as Tawila and eastern Jebel Marra [1292][1319]. Fuel shocks and global shipping delays have increased transport and procurement costs for traders and humanitarian actors, eroding the purchasing power of cash assistance — which covered about 43% of a standard food basket by March 2026 — and reducing operational reach [1304]. Humanitarian assistance is insufficient to fill market gaps: only about 20% of Sudan's 2026 Humanitarian Needs and Response Plan had been funded by April 2026, and partners aimed to reach 4.8 million people per month between February–May but only an estimated 3.13 million received assistance in February, limiting aid's stabilising effect on availability and affordability [1307][1300]. As a result, food availability and affordability are expected to worsen in the upcoming June–September lean season, with parts of Greater Darfur, Greater Kordofan and Blue Nile at highest risk of Emergency or worse outcomes [1303][1302].

Coping mechanisms

Families have adopted extreme negative coping strategies including eating wild foods, consuming seeds reserved for planting, animal feed, leaves, and, in some reports, breaking into slaughterhouses to obtain food [1323][1292]. Farming households widely reduced agricultural investment to free resources for food, with over 60% cutting input spending across most surveyed states and roughly 70% doing so in West Kordofan, Blue Nile, and South Kordofan [1284]. Sale of productive assets and transport is common in hardest-hit areas – more than half of farming households in West Kordofan and Khartoum reported selling productive assets or means of transport, while roughly 50% of households in South Kordofan and White Nile sold household assets to buy food; in contrast, fewer than 40% did so in Kassala, Northern, and Blue Nile [1284]. Coping is often multi-layered: about 30% of farming households relied on a single strategy, while a significant share in Kordofan, White Nile, and Al Jazirah reported using two or all three surveyed coping measures [1284]. Protection analyses report that asset depletion and acute deprivation are driving harmful long-term coping mechanisms – including child labour, survival sex and sexual exploitation, early or forced marriage, family separation, and recruitment by armed actors – and that sexual exploitation as a survival strategy is increasingly documented and normalised in some settings [1338][1337][1329]. Households are also moving as a coping strategy, displacing internally toward urban centres or crossing borders (e.g., to Chad) to seek food and resources, though such movements are increasingly constrained by insecurity and access restrictions; recent displacement waves include over 28,000 people from parts of Blue Nile between January and April 2026 [1301][1298][1304]. Displaced and conflict-affected households are heavily reliant on limited humanitarian assistance and community support to survive, and partners warn that constrained access and funding shortfalls are worsening needs and forcing more severe coping behaviours [1292][1299][1302].

Food security needs and priorities

FAO, WFP and UNICEF emphasise immediate scale-up of lifesaving food assistance, rapid expansion of emergency agricultural support (seeds, tools and inputs) to restore local production, scaling up treatment and screening for acute malnutrition, and urgent reinforcement of WASH and health services alongside unhindered humanitarian access and protection of civilians [1307][1300][1298]. The Sudan 2024 Smallholder Farmers Survey shows food assistance was the top priority for farming households (42%); state-specific priorities include food assistance and security measures for South and North Kordofan, water and health services for Red Sea and Kassala, and agricultural inputs for AlJazirah and River Nile [1284]. Partners planned to target about 4.8 million people per month (February–May) and roughly 4.4 million per month (June–September) but face major access and funding constraints, with WFP reaching roughly 3.1 million people with food and cash in February – below planned monthly targets [1300][1292]. FEWS NET and IPC identify highest-risk localities where immediate, sustained assistance and access are critical over the June–September lean season, including Um Baru, Kernoï and AtTine in northwestern North Darfur, parts of South Kordofan (Kadugli, Dilling and surrounding areas), and displaced populations around ElObeid [1303][1292].

Shelter & NFI

Damage to housing and residential infrastructure

Shelter needs are critical ahead of the rainy season, with widespread housing destruction compounding an already severe displacement crisis. IOM reporting notes that some 70% of homes to which people returned in Khartoum were partially or totally destroyed, and that over 500,000 people around Tawila face extremely high levels of homelessness (reported >85% severely homeless) [1345]. Repeated fires and camp incidents have caused acute shelter losses: 24 fires in March destroyed 1,571 shelters, 76 latrines and WASH infrastructure in Tawila, affecting 7,855 people [1349], while separate fires in East and South Darfur recently destroyed about 62 homes in AlMuhajir and over 60 homes in AlSalam displacement camp [1373]. Damage to communal infrastructure—including markets, water systems and health facilities—has been severe: roughly 40% of health facilities are nonfunctional and multiple hospitals and health warehouses have been struck, further undermining prospects for safe return and rebuilding [1348][1366]. Rebuilding is constrained by insecure land tenure, overcrowding, limited construction materials, constrained humanitarian access and severe funding shortfalls [1356][1355][1344]. Cluster prioritization highlights floodprone localities in Darfur (North, South, Central, East), Gedaref, Northern, River Nile, White Nile, Blue Nile and specific displacement hubs (including parts of Aj Jazirah, Kassala, Red Sea and Kordofan states) as priority locations for shelter and sitesafety interventions [1355][1344]. Preparedness guidance calls for pre-positioning, site-level disaster risk reduction (DRR), shelter reinforcement and rapid shelter response within 24–72 hours of verified flood events to protect IDP sites and collective centres before peak rains [1367][1355].

Camp and displaced populations

Displacement sites across Sudan are overcrowded and lack adequate shelter, WASH, lighting and privacy, increasing protection and publichealth risks for women, children, older persons and people with disabilities [1355][1346][1366]. WASH coverage gaps are acute—in Tawila, for example, only 52% of IDPs have access to clean water, 83% of households lack soap, 52% practice open defecation and 42% of latrines are unusable—heightening disease and nutrition risks ahead of the rains [1349][1348]. In Blue Nile, displacement sites faced severe shortages, overcrowding and rising disease risk in May 2026 after authorities suspended emergency committees, and IOM reported 49,512 displaced in the state between 11 January and 4 May 2026, representing a 77% increase versus the previous period [1377]. Protection threats in sites include elevated GBV, exploitation, theft, forced eviction, family separation and child recruitment, compounded by limited lighting, unsafe routes to services and exclusion of persons with disabilities, as emphasized in Protection and Site Management Cluster guidance [1354][1356][1355]. Flood susceptibility analyses show major overlaps with IDP concentrations: approximately 6.91 million IDPs are exposed to mediumtoveryhigh flood risk across 183 localities (1,220 mapped IDP sites), and over 4 million are exposed to high or very high flood susceptibility across 163 localities covering 1,195 IDP sites, making rainyseason DRR, shelter reinforcement and sitesafety measures protectioncritical [1346][1344][1355].

Site-level risks—including flooding, waterlogging, wind damage and fire—will amplify protection and public health impacts if shelters fail [1355][1346]. Humanitarian access constraints and underfunding are hindering prepositioning, DRR and rapid shelter/NFI responses ahead of the rainy season, placing millions of IDPs and returnees at heightened risk [1346][1344][1348].

Shelter NFI needs and priorities

The ES/NFI Cluster reports a critical 2026 funding gap: only US\$35.3 million of US\$179.4 million required is secured (19.7% coverage), severely undermining rainy-season preparedness [1344][1346][1355]. The 2026 Humanitarian Needs and Response Plan (HNRP) identifies 15.4 million people in need of emergency shelter and NFI assistance, while current plans target only 1.8 million and prioritize 1.6 million due to funding constraints [1344][1346]. Key planned cluster interventions and estimated costs include:

- Emergency shelter assistance: 33,187 ESK households — US\$28.99M [1346].
- Shelter repair/reconstruction toolkits: 10,077 households — US\$7.14M [1346].
- NFI / EHI kits: 272,077 households — US\$59.41M [1346].
- Collective site rehabilitation: 34,297 people — US\$18.67M [1346].
- Settlement infrastructure improvements: 61,558 households — US\$33.60M [1346].
- Street lighting (site safety): 61,558 households — US\$16.80M [1346].

The Site Management Cluster and the joint Shelter/NFI Preparedness and Response Plan (May 2026) call for pre-positioning ES/NFI kits, funding low-cost DRR and site-safety measures (platform elevation, drainage clearing, protective barriers, lighting, safer access routes), prioritizing collective site rehabilitation, and integrating protection-sensitive design with WASH, Health, Protection and CCCM actors [1355][1367]. The Cluster warns that shelter failure increases protection risks including GBV, exploitation, family separation, eviction, disease exposure and exclusion of persons with disabilities [1346][1344]. Field-level distributions in May 2026 include Qatar Charity's 11 May convoy to Khartoum delivering 1,000 blankets and 500 personal hygiene kits among other NFIs to approximately 400 displaced families [1379]. Sudanese NGO SI reported distributing ES/NFI and hygiene kits to returnees and IDPs in early 2026, including 1,491 ESNFI kits restoring access for about 10,491 people and targeted ES/NFI/hygiene distributions to returnee households in Abu Digais, Arab Bashir and Ammar Jadeed, amid repeated camp fires and access challenges [1349]. Evaluations of the Common Pipeline mechanism show benefits for speed, quality and prepositioning of shelter/NFI stocks—with large lead-time reductions and cost efficiencies—but localization, formal governance, and high reliance on international sourcing remain limitations to sustaining NFI availability at scale [1384]. Operational constraints including severely limited humanitarian funding (the 2026 HNRP was approximately 20% funded as of April 2026), insecure access and logistics disruptions continue to restrict distributions and force strict prioritization of beneficiaries [1348][1347][1384]. Urgent scaling of donor funding, pre-positioned ES/NFI stocks, and protected financing for rainy-season DRR and site-safety measures is required to reduce protection and public-health risks for the 15.4 million people identified in the HNRP [1344][1346][1348].

Livelihoods & Agriculture

Impact on livelihoods and household incomes

Sudan's conflict, economic instability, and climate shocks have sharply contracted household incomes and undermined farming livelihoods, with severe asset losses — including declining livestock due to forced sales, theft and looting, contraction of cultivated land, and destruction of markets and agricultural assets — further constraining production and income sources [1391][1410]. Households have shifted from farming toward non-agricultural businesses, casual labour and reliance on humanitarian assistance, while the share reporting no income or employment rose sharply, with near-universal income losses reported in states such as Sennar and parts of Kordofan [1391]. Sharp declines in key cash crops — sesame and groundnuts falling 48% and 55% below the pre-war five-year average respectively — alongside lower national cereal output have contributed to further income loss for agricultural households [1418]. By March 2026, humanitarian cash transfers covered only about 59% of a standard food basket, falling to roughly 43% thereafter, reducing the effective value of assistance to households [1398].

Barriers to farming and agriculture

Conflict-driven displacement, asset losses, reduced cultivated land and declining livestock holdings have undermined household resilience and sharply contracted farming incomes and production capacity [1391][1418]. A large share of farmers indicated they did not plan to cultivate during the 2024 summer season, reflecting disrupted farming decisions and reduced agricultural engagement [1391]. Smallholder farmers also reported high input costs: 47% cited high seed prices as the main seed-access challenge during the 2024 summer season, and high fertilizer prices were the most common constraint to accessing fertilizer, with only 20% reporting no fertilizer difficulties [1391]. Widespread destruction of civilian infrastructure — including markets, water points, health facilities and agricultural assets — together with siege-like access restrictions, transport bans and localized market closures are directly constraining production, market access, and the delivery of inputs and assistance, increasing the risk of crop and livestock losses in affected states [1409][1410][1420]. External supply shocks linked to the Middle East crisis have lengthened shipping times (up to approximately 25 days), raised fuel costs and tightened fertilizer availability — Sudan imports roughly 54% of its fertilizer from Gulf countries — exacerbating input shortages and on-farm costs [1398][1394]. National cereal production for 2025/26 is estimated at 5.2 million tonnes, about 22% below the previous year and roughly 27% below the pre-war five-year average, reflecting reduced field access, input shortages, limited agricultural finance and disruptions to harvesting and markets [1418].

Humanitarian agricultural support remains inadequate, with only about 20% of Sudan's 2026 Humanitarian Needs and Response Plan funded as of April 2026 [1410][1412].

Livelihood agriculture needs and priorities

FAO, WFP and UNICEF on 15 May 2026 called for an urgent scale-up of emergency agricultural assistance — including seeds, tools and inputs — to boost local food production and reduce dependence on aid [1410][1412]. Recommended priorities include strengthening humanitarian assistance and social protection (food aid, cash transfers and livelihood assistance), revitalizing agricultural input and finance systems (restoring access to quality seed, fertilizer, pesticides and irrigation through targeted subsidies and microfinance), and protecting livestock and livelihood assets through restocking programs, veterinary support, feed provision and safeguarding of grazing routes [1391][1405]. Humanitarian actors and the Food Resources Commission have urged protection of farms, markets, water and health infrastructure and unhindered humanitarian and commercial access to prevent further livelihood collapse, particularly before the rainy season renders routes impassable [1409][1410]. Thematic risk analyses further warn that transport bans, market disruptions and power and irrigation outages are restricting access to agricultural inputs and irrigation schemes, threatening crop and livestock production and household incomes ahead of the planting season [1420].

Protection

Attacks on civilians

According to the UN Human Rights Office and UN human rights chief Volker Türk, armed drones killed at least 880 civilians between January and April 2026 and accounted for more than 80% of conflict-related civilian deaths in that period [1508]. Most deadly drone strikes were recorded in Kordofan and Darfur, and strikes on 8 May killed 26 civilians in Al Quoz (South Kordofan) and near El Obeid (North Kordofan) [1508]. The UN reported at least 28 attacks on markets and 12 strikes on health facilities over the same four-month period, with some hospitals forced to close and supply routes and aid trucks destroyed, further limiting access to care [1508]. Protection Cluster analysis notes that attacks on civilian objects — markets, hospitals, schools, water infrastructure and IDP sites — are depriving communities of healthcare, education, water and humanitarian services, increasing displacement, family separation, food insecurity and exposure to further harm [1462]. WFP, FAO and UNICEF warned that these attacks and broader destruction of civilian infrastructure have constrained food production and humanitarian access, with close to nine million people internally displaced as of end March 2026 and around 40% of health facilities nonfunctional [1457].

Gender based violence (GBV)

A January 2026 PSEA strategic analysis estimated that over 12.2 million people in Sudan face GBV risk — an 80% increase from 2024 and roughly 350% since the war began — and ranked Sudan fourth on the SEARO Global Index for SEA risk as of December 2025 [1469]. Protection Cluster reporting from March 2026 states GBV is widespread, systematic and severely underreported, with conflict-related sexual violence (including rape, gang rape, sexual slavery), sexual exploitation, forced marriage and intimate partner violence reported in displacement sites, host communities, transit routes, checkpoints and besieged areas, while survivors frequently lack clinical care, psychosocial support and safe referral pathways [1462][1465]. UN investigators and UN Special Rapporteur Reem Alsalem have documented that rape is being used as a weapon of war, highlighting particularly widespread and systematic abuses by RSF in Darfur and Kordofan, with thousands of survivors and numerous pregnancies resulting from rape reported by UN field officials [1484][1472]. MSF and other health actors report sexual violence as a feature of the conflict and note repeated attacks on health facilities and severe constraints on access to sexual and reproductive health and trauma services across Darfur and other frontline states [1502][1474]. Mid-May analyses warn that expanding drone warfare, escalating frontlines and obstructed humanitarian access are driving heightened GBV and protection risks among newly displaced populations in Blue Nile, Kordofan and Darfur [1463][1488]. Humanitarian assessments note severe service and funding gaps: UNFPA reported a 90% funding gap against its Chad response requirement, and UNFPA-led safety audits found 96% of sites lacked night-time lighting and 60% of water points were non-functional, increasing exposure of women and girls to assault during daily tasks [1473].

Factors driving protection risks

The protection environment in Sudan is driven by a convergence of overlapping and mutually reinforcing risks. Attacks on civilians and civilian objects — markets, hospitals, schools, IDP sites, roads and water infrastructure — remain widespread in 2026, destroying services and driving displacement [1462][1465]. UN and African investigators have documented increasing arbitrary arrests, enforced disappearances, torture in secret detention centres and the targeted detention or killing of professionals, undermining civic institutions and access to justice [1503]. Explosive ordnance contamination, including UXO, ERW and newly confirmed landmines in areas previously considered UXO-only (including parts of Khartoum), has escalated and is severely restricting movement, livelihoods and prospects for safe return [1465][1461]. Forced recruitment and use of children, abduction and family separation have increased, with UNICEF reporting at least 42,000 unaccompanied and separated children registered in Sudan and neighbouring countries [1462]. Widespread theft, looting, extortion, forced evictions and destruction of civilian property, combined with economic collapse, rising food and fuel prices and disrupted markets, have depleted household assets and driven people into exploitative coping strategies including survival sex, child labour and trafficking [1462][1477]. Discrimination and stigmatization — based on ethnicity, perceived affiliation with parties to the conflict, displacement status, gender, disability and lack of civil documentation — are limiting access to assistance, services and legal protection [1462][1463]. Humanitarian access is severely constrained by siege-like conditions, bureaucratic impediments and attacks on aid personnel, reducing protection and PSEA capacity; in 2025, forty-four aid workers were killed, 22 kidnapped and 19 wounded, further weakening protection delivery [1463][1469]. Shelter failure and rainy-season risks place IDP populations at heightened protection risk: about 6.91 million IDPs are exposed to medium-to-very-high flood susceptibility and over 4 million to high or very high exposure, exacerbating GBV, family separation and WASH-related harms [1468]. Regional spillover and mass cross-border displacement — illustrated by 926,421 registered arrivals from Sudan into Chad as of 26 April 2026 — are straining host services and amplifying protection vulnerabilities among refugees and displaced populations [1473][1470].

Impact on mental health

Repeated drone strikes, attacks on health facilities and largescale displacement have produced widespread trauma, anxiety and psychosocial distress across Sudan in 2026 [1509][1508]. Mental health and psychosocial support (MHPSS) services are severely constrained by the nearcollapse of the health system – an estimated 40% of health facilities were nonoperational as of January 2026 – as well as attacks on hospitals, restricted humanitarian access and protection funding shortfalls [1509][1492][1469]. Barriers to care include destroyed facilities and supply chains, siegelike movement restrictions and denials of access, widespread stigma that suppresses reporting of sexual violence, and limited integration of MHPSS into health cluster planning [1492][1461][1502][1533]. Households are adopting harmful coping mechanisms – including child labour, survival sex, family separation and unsafe movement – which both reflect and deepen psychosocial distress among displaced communities [1461][1521]. Children face elevated protection and mental health risks from forced recruitment, abduction and family separation, while an estimated 825,000 children under five are expected to suffer severe acute malnutrition in 2026, compounding psychological vulnerability [1461][1509][1465]. Women and girls face high levels of sexual violence but reporting and services remain limited, and older persons and people with disabilities among the displaced experience heightened psychosocial harm [1502][1509].

Protection needs and priorities

The protection environment in Sudan remains acute and multidimensional. Protection and GBV services are severely underfunded: the Protection Cluster received roughly 12.5% of requirements and GBV interventions are under 15% funded, while the 2026 Humanitarian Response Plan is only about 20% funded overall [1469][1492]. Emergency shelter and NFI coverage is critically low: the ES/NFI Cluster has secured US\$35.3 million of the required US\$179.4 million (approximately 19.7% coverage), with 15.4 million people identified as needing ES/NFI assistance but only 1.8 million targeted and 1.6 million prioritized under current constraints [1476]. Flood and rainy-season exposure magnifies protection needs: approximately 6.91 million IDPs are exposed to medium-to-very-high flood risk across 183 localities (1,220 mapped IDP sites), with over 4 million at high or very high risk in 163 localities, raising risks of GBV, exploitation, family separation and public-health threats [1476]. Shelter failures – lack of privacy, lighting, safe access to latrines and inaccessible designs – directly increase protection harms for women, girls, older persons and people with disabilities, requiring protection-sensitive site planning, disaster risk reduction, and a protected minimum response package ahead of peak rains [1468][1481][1511]. Humanitarian access, monitoring, case management and referral systems are constrained by armed-group fragmentation, threats to aid workers and territorial control, increasing reliance on local civil society and mutual-aid groups and undermining consistent protection monitoring and referrals, according to ACAPS risk analysis in May 2026 [1463][1480]. Operational efforts to sustain protection capacity were scaled up regionally, including ICRC's start of RED aircraft operations from Jeddah on 14 May 2026 to accelerate deployment of humanitarian personnel into Sudan [1546]. Cross-border displacement continues to exacerbate protection gaps: as of 26 April 2026, Chad had registered 926,421 arrivals from Sudan (87% women and children), and a UNFPA-led GBV safety audit in Lac Province found 96% of sites lacked nighttime lighting, 60% of water points were non-functional and 68% of residents lacked latrines – conditions that heighten risks for women and girls and complicate safe case identification and referrals [1473][1500]. UNICEF and partners emphasize the urgent need to strengthen case management, family-tracing and referral systems for unaccompanied and separated children amid persistent access and resource constraints [1490].

Operational Environment

Economy

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